Reach-Avoid Constrained Selection for Safe Counterfactual Intervention Planning

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Abstract

Variational counterfactual intervention planning selects treatment sequences by optimizing an evidence lower bound (ELBO) toward a point target outcome. In clinical practice, however, the goal is typically to bring a patient's state into an *acceptable range* while maintaining *safety constraints* at every intermediate step—requirements that point-target optimization does not address. We introduce *reach-avoid constrained selection*, a post-hoc method that filters ELBO-ranked candidate sequences through a reach-avoid feasibility criterion: the terminal outcome must lie within a target set $\mathcal{T}$ and all intermediate outcomes must remain within a safety set $\mathcal{S}$. We prove that reach-avoid ranking is more robust to variational approximation error than point-target ELBO ranking, with an explicit bound relating the ranking preservation margin to the variational gap $\varepsilon_{\mathrm{VI}}$ and soft-indicator approximation error $\varepsilon_{\mathrm{soft}}$. We further provide a sensitivity analysis under hidden confounding, showing that the safety guarantee degrades gracefully as the Rosenbaum sensitivity parameter Γ increases, with non-vacuous bounds through $\Gamma = 2.0$ at all tested horizons. Empirically, on a cancer tumor growth simulator with known counterfactuals, reach-avoid constrained selection improves safety by 18–33 percentage points under moderate-to-strong confounding at modest ranking cost, Pareto-dominates soft-constraint alternatives, and transfers to all baseline models without retraining. On MIMIC-III intensive care data, the method lifts in-target diastolic blood pressure rates from 73–81% to 99–100%. To our knowledge, this is the first systematic safety evaluation of counterfactual treatment planners. The approach requires no model modification, no retraining, and is compatible with any variational counterfactual planner.

1 Introduction

Individualized treatment planning from observational data is a central challenge in precision medicine [Chakraborty and Moodie, 2014, Murphy, 2003]. Given a patient's clinical history, the goal is to recommend a sequence of interventions—medications, dosages, ventilation settings—that will produce the best outcome over a planning horizon τ. Recent advances in variational latent-dynamics models have made this tractable: Variational Counterfactual Intervention Planning [VCIP; Melnychuk et al., 2025] learns a latent state-space model from observational trajectories and selects treatment sequences by optimizing an evidence lower bound (ELBO) toward a target outcome value.

However, clinical treatment planning has two features that point-target ELBO optimization does not capture:

Range targets, not point targets. Clinicians do not aim for a single number. They aim for an *acceptable range*: diastolic blood pressure in $[60, 90]$ mmHg, tumor volume below a stage-dependent threshold, blood glucose within safe bounds. A treatment plan that overshoots the target by 1% and

Submitted to 40th Conference on Neural Information Processing Systems (NeurIPS 2026). Do not distribute.

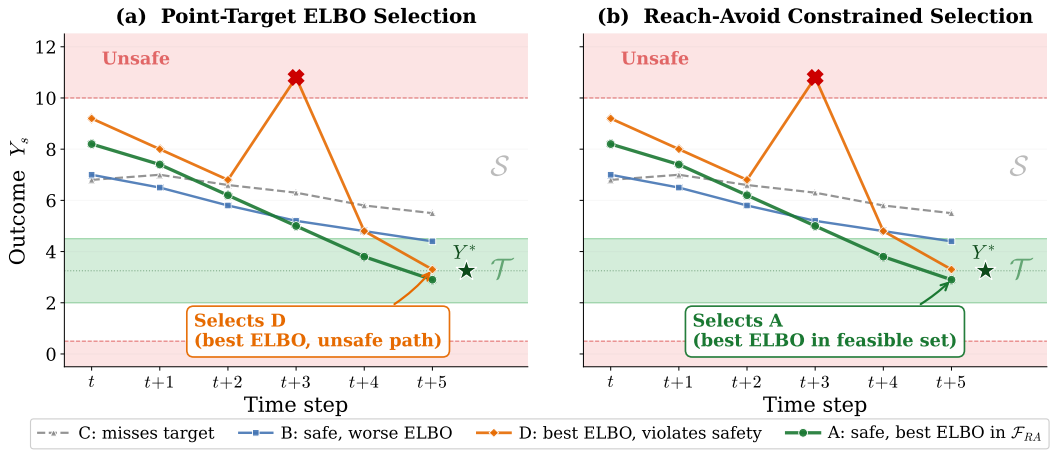

Figure 1: **Reach-avoid vs. point-target selection.** The dark green star (★) marks the ELBO point target Y^*. Left: ELBO-based selection picks sequence D (closest to Y^* at terminal time) despite its safety violation at $t{+}3$. Right: RA-constrained selection restricts to feasible sequences (those reaching $\mathcal{T}$ while staying in $\mathcal{S}$) and selects the best ELBO among them (A over B, which is also feasible but has worse ELBO). D is rejected for violating safety; C for missing the target.

one that undershoots it by 50% receive the same ELBO penalty if they are equidistant from the point target, yet their clinical implications differ fundamentally. This limitation was explicitly identified in the VCIP peer review process, where a reviewer noted the restrictive nature of single-point targets for clinical applications and the authors acknowledged range-based targets as important future work.

Path safety, not just terminal achievement. ELBO-based planning evaluates only the terminal outcome $Y_{t+\tau}$. It is blind to what happens along the way. A chemotherapy regimen that produces dangerously high toxicity at step $t{+}3$ before recovering at step $t{+}\tau$ may rank identically to a safe regimen—both achieve the same terminal tumor volume. In clinical practice, intermediate safety violations are unacceptable: a patient whose blood pressure drops to dangerous levels during treatment has suffered harm regardless of the final reading.

These concerns arise naturally from the reach-avoid control literature [Mitchell et al., 2005, Fisac et al., 2015], where the objective is to steer a dynamical system into a target set while avoiding unsafe regions throughout the trajectory. Figure 1 illustrates the distinction: ELBO-based selection picks the sequence with the best terminal loss (D), ignoring its unsafe intermediate path, while reach-avoid constrained selection picks the best ELBO among sequences with safe trajectories (A).

Our approach. We propose *reach-avoid constrained selection*: given a set of candidate treatment sequences ranked by ELBO, we filter out sequences that fail a reach-avoid feasibility criterion and select the best ELBO among the feasible set:

$$\bar{a}^* = \arg\min_{\bar{a} \in \mathcal{F}_{\mathrm{RA}}} \mathrm{ELBO}(\bar{a}), \tag{1}$$

where $\mathcal{F}_{\mathrm{RA}} = \{\bar{a} : Y_{t+\tau}[\bar{a}] \in \mathcal{T} \text{ and } Y_s[\bar{a}] \in \mathcal{S} \text{ for all } s = t, \ldots, t{+}\tau\}$ is the set of reach-avoid feasible sequences. This is deliberately simple: no new model, no retraining, no gradient-based planning. The method operates as a *post-hoc safety filter* on any existing variational counterfactual planner.

The simplicity is a feature, not a limitation. By separating safety (via the RA filter) from quality (via ELBO ranking), we avoid the failure mode we observed when attempting to use RA scoring as a *replacement* for ELBO ranking: the RA score is near-binary (sequences are either clearly feasible or clearly not), providing no discrimination among candidates. ELBO ranking and RA feasibility are complementary—each does what the other cannot. Any practitioner building a clinical decision support system would add safety checks; our contribution is the *formalization* that makes this principled and the *systematic evaluation* revealing when and why it matters.

Contributions.

1. **First systematic safety evaluation of counterfactual planners.** We formalize reach-avoid constrained intervention planning, separating safety from quality in a way that reveals a previously unquantified safety gap: existing planners violate clinically meaningful constraints on 15–45% of patients under moderate confounding (Section 6).
2. **Ranking robustness theory.** We prove that reach-avoid ranking is more robust to variational approximation error than point-target ELBO ranking (Theorem 1), with an explicit, empirically non-vacuous bound ($\varepsilon_{\mathrm{VI}} \approx 0.09$–$0.23$) on ranking preservation (Section 5).
3. **Sensitivity analysis under hidden confounding.** We extend the Rosenbaum sensitivity model to sequential treatments and prove that the RA safety guarantee degrades gracefully as hidden confounding increases (Theorem 5), with non-vacuous bounds through $\Gamma = 2.0$ at all tested horizons and explicit breakdown points $\Gamma_{\max} = 2.1$–7.5 (Section 5.2).
4. **Comprehensive experiments.** On the Cancer simulator, RA-constrained selection yields +18–33 pp safety improvement under moderate-to-strong confounding, Pareto-dominates soft-constraint alternatives, and generalizes across five baseline models without retraining. On MIMIC-III ICU data, it lifts in-target rates from 73–81% to 99–100% (Section 6).
5. **Model-agnostic post-hoc method.** The approach requires no retraining—we show empirically that RA-aware retraining produces identical results to post-hoc filtering—and is compatible with any variational counterfactual planner (Section D).

2 Related Work

We provide a concise overview here; Appendix C contains a comprehensive survey with additional references.

Counterfactual outcome estimation and planning. A growing body of work estimates individualized treatment effects from longitudinal observational data, including balanced representations [Shalit et al., 2017, Bica et al., 2020a], variational approaches [Louizos et al., 2017], attention-based architectures [Melnychuk et al., 2022], recurrent methods [Lim, 2018, Li et al., 2021], continuous-time models [Seedat et al., 2022], and synthetic control [Qian et al., 2021]. Recent advances address hidden confounding [Bica et al., 2020b, Frauen et al., 2023] and employ contrastive [Kim et al., 2024] or generative [Wu et al., 2024, Pawlowski et al., 2020] approaches. These methods estimate outcomes for *given* treatments but do not optimize over treatment sequences. VCIP [Melnychuk et al., 2025] extends this line to *planning*: it selects intervention sequences by ELBO minimization toward a point target. Our work keeps the VCIP planning infrastructure intact and adds a safety-aware selection layer.

Dynamic treatment regimes. The DTR literature [Murphy, 2003, Chakraborty and Moodie, 2014, Hernán and Robins, 2020] formalizes sequential treatment decisions under the potential outcomes framework, with recent work on when-to-treat policies [Nie et al., 2021] and batch constrained learning [Le et al., 2019]. Most DTR methods learn decision rules mapping states to actions, optimizing expected outcomes over the full trajectory. In contrast, VCIP and our extension operate in a *counterfactual planning* mode: given a specific patient history, they evaluate and rank complete intervention sequences. The reach-avoid formulation adds the constraint that trajectories must remain safe—a concern shared with constrained DTR work but addressed here through a distinct mechanism (post-hoc filtering rather than constrained policy learning).

Safe and constrained sequential decision-making. Safe reinforcement learning [García and Fernández, 2015, Achiam et al., 2017, Tessler et al., 2019] constrains policies to satisfy safety specifications during execution, from Lyapunov-based methods [Chow et al., 2018] to safety layers [Dalal et al., 2018] to offline safe RL [Zheng et al., 2024]. These methods operate in the online or offline RL setting, requiring either environment interaction or fixed datasets of state-action trajectories. Our setting is fundamentally different: we select among candidate plans evaluated on a learned model, with no opportunity for policy-level correction. This distinction motivates a filter-based approach rather than constrained policy optimization. The *reach-avoid* concept from control theory [Mitchell et al., 2005, Fisac et al., 2015, Bansal and Tomlin, 2021] formalizes the objective of reaching a target set while avoiding unsafe regions. Chance-constrained optimization [Nemirovski and Shapiro, 2006] and robust MDPs [Iyengar, 2005] address related problems under uncertainty. We import the reach-avoid concept into the variational counterfactual planning framework, replacing the continuous reachability computation with a discrete feasibility check over candidate sequences.

Variational causal inference. CEVAE [Louizos et al., 2017] pioneered VAE-based causal inference under hidden confounding; Deep SCMs [Pawlowski et al., 2020] extended this to full counterfactual reasoning via normalizing flows. VCI [Wu and Bareinboim, 2025] derives an ELBO for individual-level counterfactual likelihoods in a static setting, introducing latent disentanglement via $D_{\mathrm{KL}}[q(z|y,t,x)\|q(z|y',t',x)]$. We draw on VCI's insight that latent action-sensitivity is a diagnostic for counterfactual model quality (Section E), adapting it to the sequential setting. Our diagnostic reveals that VCIP's latent space is nearly action-invariant ($D_{\mathrm{KL}} \approx 10^{-5}$), with action-outcome coupling residing entirely in the decoder.

3 Background: Variational Counterfactual Intervention Planning

We briefly review the VCIP framework [Melnychuk et al., 2025] to establish notation.

Setting. Consider a longitudinal observational dataset with patient histories $\bar{H}_t = (X_1, A_1, Y_1, \ldots, X_t, A_t, Y_t)$, where $X_s \in \mathbb{R}^p$ are time-varying covariates, $A_s \in \mathcal{A}$ are treatments (binary or categorical), and $Y_s \in \mathbb{R}^d$ are outcomes of interest. Given a patient's history $\bar{H}_t$ up to time t and a planning horizon τ, the goal is to select an intervention sequence $\bar{a} = (a_{t+1}, \ldots, a_{t+\tau}) \in \mathcal{A}^\tau$ that optimizes the counterfactual outcome $Y_{t+\tau}[\bar{a}]$.

Latent dynamics model. VCIP posits a latent state $Z_s \in \mathbb{R}^{d_z}$ with:

- Generative transition: $p_\theta(Z_{s+1} \mid Z_s, a_s)$, modeling how latent states evolve under treatment,
- Emission (decoder): $p_\theta(Y_s \mid Z_s)$, mapping latent states to predicted outcomes,
- Variational posterior: $q_\phi(Z_{t:t+\tau} \mid \bar{H}_t, \bar{a})$, approximating the intractable posterior over latent trajectories.

The model is trained by maximizing an ELBO on observed trajectories:

$$\mathcal{L}(\theta,\phi) = \mathbb{E}_{q_\phi}\left[\sum_{s=t+1}^{t+\tau} \log p_\theta(Y_s \mid Z_s)\right] - D_{\mathrm{KL}}\left[q_\phi(Z_{t:t+\tau} \mid \bar{H}_t, \bar{a}) \,\|\, p_\theta(Z_{t:t+\tau} \mid \bar{H}_t, \bar{a})\right]. \quad (2)$$

Planning by ELBO minimization. Given a patient history $\bar{H}_t$, VCIP generates k candidate treatment sequences (via perturbation of the observed treatments) and selects the one minimizing the ELBO loss with respect to a point target Y^*:

$$\bar{a}^*_{\mathrm{ELBO}} = \arg\min_{\bar{a}\in\{\bar{a}_1,\ldots,\bar{a}_k\}} \mathrm{ELBO}(\bar{a}; \bar{H}_t, Y^*). \quad (3)$$

The ELBO loss decomposes into a reconstruction term (how well the predicted outcome matches Y^*) and a KL regularization term. This formulation ranks sequences by how well the variational model predicts they will achieve the point target. The key limitation is that Y^* is a single point, and only the terminal step $s = t + \tau$ contributes to the ranking—intermediate outcomes are ignored.

4 Method: Reach-Avoid Constrained Selection

4.1 Reach-Avoid Feasibility

Definition 1 (Reach-Avoid Event)**.** *Given a target set $\mathcal{T} \subseteq \mathbb{R}^d$ for the terminal outcome and a safety set $\mathcal{S} \subseteq \mathbb{R}^d$ for intermediate outcomes, the* reach-avoid event *for intervention sequence $\bar{a}$ is:*

$$E(\bar{a}) := \{Y_{t+\tau}[\bar{a}] \in \mathcal{T}\} \cap \bigcap_{s=t}^{t+\tau} \{Y_s[\bar{a}] \in \mathcal{S}\}. \quad (4)$$

The event $E(\bar{a})$ requires the terminal outcome to land in the target set (*reach*) while all intermediate outcomes remain in the safety set (*avoid* unsafe regions). The sets $\mathcal{T}$ and $\mathcal{S}$ encode domain knowledge:

- **Cancer:** $\mathcal{T} = \{y : \text{tumor volume} \leq 3.0\}$ (clinically manageable stage), $\mathcal{S} = \{y : \text{volume} \leq 12.0\} \cap \{y : \text{chemo dosage} \leq 5.0\}$ (no tumor explosion, no toxic chemotherapy).
- **ICU (MIMIC-III):** $\mathcal{T} = \{y : \text{DBP} \in [60, 90] \text{ mmHg}\}$ (normotensive range), $\mathcal{S} = \{y : \text{DBP} \in [40, 120] \text{ mmHg}\}$ (avoids dangerous hypo-/hypertension).

Algorithm 1 Reach-Avoid Constrained Selection

Require: Patient history $\bar{H}_t$, trained model (p_θ, q_ϕ), target set $\mathcal{T}$, safety set $\mathcal{S}$, candidates k, hardness κ, threshold δ

Generate k candidate sequences $\{\bar{a}_1, \ldots, \bar{a}_k\}$ via treatment perturbation
Compute $\mathrm{ELBO}(\bar{a}_i; \bar{H}_t, Y^*)$ for each i using the variational model
Obtain outcome trajectories $\{Y_s[\bar{a}_i]\}_{s=t}^{t+\tau}$ for each i {simulator or model predictions}
Compute RA scores: $J_{\mathrm{RA}}(\bar{a}_i) \leftarrow g_{\mathcal{T}}(Y_{t+\tau}[\bar{a}_i]) \cdot \prod_s g_{\mathcal{S}}(Y_s[\bar{a}_i])$
Filter: $\mathcal{F}_{\mathrm{RA}} \leftarrow \{\bar{a}_i : J_{\mathrm{RA}}(\bar{a}_i) \geq \delta\}$
if $\mathcal{F}_{\mathrm{RA}} \neq \emptyset$ **then**
  return $\bar{a}^* = \arg\min_{\bar{a} \in \mathcal{F}_{\mathrm{RA}}} \mathrm{ELBO}(\bar{a})$
else
  return $\bar{a}^* = \arg\min_{\bar{a} \in \{\bar{a}_1, \ldots, \bar{a}_k\}} \mathrm{ELBO}(\bar{a})$ {fallback}
end if

4.2 Soft Indicators

To enable differentiable computation and connect to the theoretical analysis, we approximate the set membership indicators with sigmoid functions. For an upper-bounded constraint $y \leq u$:

$$g_u(y; \kappa) = \sigma(\kappa(u - y)), \tag{5}$$

where σ is the logistic sigmoid and $\kappa > 0$ controls hardness. For an interval constraint $y \in [\ell, u]$:

$$g_{[\ell,u]}(y; \kappa) = \sigma(\kappa(y - \ell)) \cdot \sigma(\kappa(u - y)). \tag{6}$$

The *reach-avoid score* for a candidate sequence $\bar{a}$ is then:

$$J_{\mathrm{RA}}(\bar{a}) = g_{\mathcal{T}}(Y_{t+\tau}[\bar{a}]) \cdot \prod_{s=t}^{t+\tau} g_{\mathcal{S}}(Y_s[\bar{a}]). \tag{7}$$

A sequence is *feasible* if $J_{\mathrm{RA}}(\bar{a}) \geq \delta$ for a threshold $\delta \in (0, 1)$. In the hard-filter limit ($\kappa \to \infty$), this reduces to the indicator $\mathbf{1}\{E(\bar{a})\}$. Our experiments show that $\kappa \geq 5$ is indistinguishable from the hard filter ($> 99.9\%$ selection agreement; see Section 6.6).

4.3 Constrained Selection

Given k candidate sequences $\{\bar{a}_1, \ldots, \bar{a}_k\}$ ranked by ELBO, the *reach-avoid constrained selection* rule is:

$$\bar{a}^* = \arg\min_{\bar{a} \in \mathcal{F}_{\mathrm{RA}}} \mathrm{ELBO}(\bar{a}; \bar{H}_t, Y^*), \quad \text{where } \mathcal{F}_{\mathrm{RA}} = \{\bar{a}_i : J_{\mathrm{RA}}(\bar{a}_i) \geq \delta\}. \tag{8}$$

If $\mathcal{F}_{\mathrm{RA}} = \emptyset$ (no feasible sequence), we fall back to unconstrained ELBO selection (3). The full procedure is summarized in Algorithm 1.

Remark 1 (Why hard filtering, not soft penalty)**.** *An alternative is Lagrangian relaxation:* $\min_{\bar{a}} \mathrm{ELBO}(\bar{a}) + \lambda(1 - J_{\mathrm{RA}}(\bar{a}))$. *We found empirically that hard filtering Pareto-dominates the soft penalty at high safety levels (Section 6.5). The key reason: a global penalty shifts rankings for* all *sequences, degrading ELBO ranking quality among safe candidates. Hard filtering is surgical—it removes infeasible sequences while preserving the ELBO ranking among the feasible set. This observation also has a theoretical basis: within $\mathcal{F}_{\mathrm{RA}}$, the ELBO ranking is unperturbed, so the constrained selection inherits the full ranking quality of the base model.*

5 Theory: Ranking Robustness

We now establish that reach-avoid ranking is provably more robust to variational approximation error than point-target ELBO ranking. This result is not mathematically deep—it follows from standard properties of total variation distance applied to bounded vs. unbounded test functions—but it serves as a *design principle*: it makes explicit *why* set-membership filtering is more reliable than point-target optimization in the presence of model error, and it provides a quantitative condition (the margin inequality below) that connects directly to the gamma-sweep results in Section 6. The key insight is that set membership is a *coarsening* of continuous outcomes: many different outcome values map to the same feasibility verdict, making the ranking less sensitive to small perturbations in the variational approximation.

Assumption 1 (Bounded variational gap)**.** *For any intervention sequence $\bar{a}$, the total variation distance between the true outcome distribution $p_\theta(\cdot \mid \bar{a}, \bar{H}_t)$ and the variational approximation $q_\phi(\cdot \mid \bar{a}, \bar{H}_t)$ is bounded:*

$$\mathrm{TV}(p_\theta(\cdot \mid \bar{a}, \bar{H}_t),\ q_\phi(\cdot \mid \bar{a}, \bar{H}_t)) \leq \varepsilon_{\mathrm{VI}}. \tag{9}$$

Assumption 2 (Bounded soft-indicator error)**.** *The soft indicator $g_{\mathcal{T}}$ approximates the true indicator $\mathbf{1}_{\mathcal{T}}$ with bounded error: $\|g_{\mathcal{T}} - \mathbf{1}_{\mathcal{T}}\|_\infty \leq \varepsilon_{\mathrm{soft}}(\kappa)$, where $\varepsilon_{\mathrm{soft}}(\kappa) \to 0$ as $\kappa \to \infty$. Similarly for $g_{\mathcal{S}}$.*

Assumption 1 is standard in variational inference analysis. Assumption 2 is constructive: for the logistic sigmoid with hardness κ, $\varepsilon_{\mathrm{soft}}(\kappa) = \sigma(0) = 0.5$ at the boundary, but this worst case occurs on a measure-zero set. We verify empirically that $\kappa \geq 5$ yields effectively zero approximation error (Section 6.6).

Definition 2 (Reach-avoid probability)**.** *For an intervention sequence $\bar{a}$, the* true reach-avoid probability *under the model is:*

$$P(\bar{a}) := \mathbb{E}_{p_\theta}\left[\mathbf{1}\{Y_{t+\tau} \in \mathcal{T}\} \cdot \prod_{s=t}^{t+\tau} \mathbf{1}\{Y_s \in \mathcal{S}\} \;\middle|\; \bar{H}_t, \bar{a}\right]. \tag{10}$$

The approximate reach-avoid probability *under the variational posterior is $\hat{P}(\bar{a})$, defined analogously with q_ϕ replacing p_θ and soft indicators replacing hard indicators.*

Theorem 1 (Ranking robustness of reach-avoid selection)**.** *Under Assumptions 1 and 2, for any two intervention sequences $\bar{a}_1, \bar{a}_2$:*

(a) RA ranking preservation. *If the true reach-avoid probability margin satisfies*

$$m_{\mathrm{RA}} := |P(\bar{a}_1) - P(\bar{a}_2)| > 2\varepsilon_{\mathrm{VI}} + 2(\tau+1)\varepsilon_{\mathrm{soft}}(\kappa), \tag{11}$$

then the approximate ranking is preserved: $\hat{P}(\bar{a}_1) > \hat{P}(\bar{a}_2)$ iff $P(\bar{a}_1) > P(\bar{a}_2)$.

(b) Point-target ranking threshold. *For point-target ELBO ranking, let $d(\bar{a}) = \mathbb{E}_{p_\theta}[\|Y_{t+\tau}[\bar{a}] - Y^*\|^2 \mid \bar{H}_t, \bar{a}]$ be the true expected squared distance to target. Ranking preservation requires:*

$$m_{\mathrm{ELBO}} := |d(\bar{a}_1) - d(\bar{a}_2)| > 2\varepsilon_{\mathrm{VI}} \cdot C_d, \tag{12}$$

where $C_d = \sup_{\bar{a}} \|Y_{t+\tau}[\bar{a}] - Y^*\|^2_{L^\infty(p_\theta)}$ is the essential supremum of the squared distance over the model's outcome support.

(c) Strict improvement. *When the boundary mass $\beta := \sup_{\bar{a}} p_\theta(Y_{t+\tau}[\bar{a}] \in \partial_\epsilon \mathcal{T} \mid \bar{H}_t, \bar{a})$ is small (most outcomes clearly inside or outside $\mathcal{T}$), we have $m_{\mathrm{RA}} \gg m_{\mathrm{ELBO}}/C_d$, i.e., RA ranking is preserved for pairs where ELBO ranking would be inverted.*

Proof sketch (full proof in Appendix A). **Part (a):** The reach-avoid probability is a bounded functional ($P \in [0,1]$). By the variational characterization of total variation, $|P(\bar{a}) - P_{q_\phi}(\bar{a})| \leq \varepsilon_{\mathrm{VI}}$. The soft-indicator approximation contributes at most $(\tau+1)\varepsilon_{\mathrm{soft}}$ error (one terminal gate plus τ safety gates). Triangle inequality gives $|\hat{P}(\bar{a}) - P(\bar{a})| \leq \varepsilon_{\mathrm{VI}} + (\tau+1)\varepsilon_{\mathrm{soft}}$. Ranking preservation follows when the margin exceeds twice this error.

Part (b): The squared distance $\|Y - Y^*\|^2$ can take large values, so the TV bound introduces the factor C_d (the essential supremum of the squared distance). In practice, C_d is large when outcomes can be far from the target—precisely the regime where point-target ranking is most fragile.

Part (c): When most outcomes are unambiguously inside or outside $\mathcal{T}$ (low boundary mass β), reach-avoid probabilities cluster near 0 or 1, giving large margins $m_{\mathrm{RA}} \approx 1$. Meanwhile, point distances can be nearly identical for outcomes on opposite sides of the target boundary, giving $m_{\mathrm{ELBO}} \approx 0$. □

Corollary 2. *In the hard-filter limit ($\kappa \to \infty$, so $\varepsilon_{\mathrm{soft}} = 0$), reach-avoid ranking is preserved whenever $|P(\bar{a}_1) - P(\bar{a}_2)| > 2\varepsilon_{\mathrm{VI}}$. For deterministic trajectories (as in the Cancer simulator), feasibility is binary ($P \in \{0,1\}$), so the filter is exact whenever $\varepsilon_{\mathrm{VI}} < 0.5$.*

Empirical verification. We estimate $\varepsilon_{\mathrm{VI}}$ on the Cancer simulator using rank mean absolute error as a proxy (Section 6.6). We find $\varepsilon_{\mathrm{VI}} \approx 0.09$–$0.23$ across confounding strengths, with ELBO pairwise ranking preservation at 61–87%. The bound is non-vacuous: for pairs differing in feasibility status (margin $= 1$), the condition $m > 2\varepsilon_{\mathrm{VI}} \approx 0.18$–$0.46$ is easily satisfied. Notably, $\varepsilon_{\mathrm{VI}}$ *decreases* with confounding strength γ (the model is better calibrated when treatment effects are stronger), meaning the bound is tightest precisely where RA filtering is most valuable.

5.1 Finite-Sample Deployment Guarantees

The ranking robustness results above are *per-patient*: they characterize when rankings are preserved for a given patient. We now provide *population-level* guarantees suitable for deployment certification (full proofs and additional results in Appendix K).

Proposition 3 (Deployment safety guarantee)**.** *Let $\{x_i\}_{i=1}^n$ be n i.i.d. patients from a population . For each patient, let $\bar{a}^*_{\mathrm{RA}}(x_i)$ be the RA-constrained selection (Algorithm 1) and let $Z_i = \mathbf{1}\{E(\bar{a}^*_{\mathrm{RA}}(x_i))\}$ indicate safety. Then with probability $\geq 1-\delta$:*

$$\mathrm{SR} \geq \widehat{\mathrm{SR}}_n - \sqrt{\tfrac{\ln(2/\delta)}{2n}}, \tag{13}$$

where $\widehat{\mathrm{SR}}_n = \frac{1}{n}\sum_i Z_i$ is the empirical safety rate and $\mathrm{SR} = \mathbb{E}_{x\sim}[Z(x)]$ is the population rate. In particular, with oracle trajectories, $Z_i = 1$ whenever $\mathcal{F}_{\mathrm{RA}}(x_i) \neq \emptyset$, so $\mathrm{SR} \geq p_F$ (the population feasibility rate) with certainty.

Proposition 4 (Constrained selection quality)**.** *Let $\eta := \sup_{\bar{a}} |\mathrm{ELBO}(\bar{a}) - \ell(\bar{a})|$ be the uniform proxy error, where ℓ is the true loss. Then the RA-constrained ELBO selection $\bar{a}^*_{\mathrm{ELBO}}$ satisfies:*

$$\ell(\bar{a}^*_{\mathrm{ELBO}}) \;\leq\; \ell(\bar{a}^*_{\mathrm{oracle}}) + 2\eta, \tag{14}$$

where $\bar{a}^*_{\mathrm{oracle}} = \arg\min_{\bar{a}\in\mathcal{F}_{\mathrm{RA}}} \ell(\bar{a})$. Total suboptimality decomposes as: $\ell(\bar{a}^*_{\mathrm{ELBO}}) - \ell(\bar{a}^*_{\mathrm{unconstrained}}) \leq \underbrace{\ell(\bar{a}^*_{\mathrm{oracle}}) - \ell(\bar{a}^*_{\mathrm{unconstrained}})}_{\textit{price of safety}} + \underbrace{2\eta}_{\textit{model error}}$.

Empirical verification. On the Cancer simulator ($n = 100$ patients, $\gamma = 4$), the RA selector achieves empirical safety $\widehat{\mathrm{SR}} = 94.4$–$97.0\%$ across horizons (including ELBO fallback for infeasible patients). Proposition 3 yields: with 95% confidence, population safety ≥ 80.8–83.4%—non-vacuous deployment bounds from only 100 patients. The quality bound is tight: empirical regret $\ell(\bar{a}^*_{\mathrm{ELBO}}) - \ell(\bar{a}^*_{\mathrm{oracle}})$ within $\mathcal{F}_{\mathrm{RA}}$ is $< 0.2\%$ of the loss range ($\ll 2\eta$), confirming that ELBO ranking quality within the feasible set is high ($\rho = 0.71$–0.89). Additional results (feasibility scaling, chance-constrained interpretation) are in Appendix K.

5.2 Sensitivity to Hidden Confounding

The preceding results assume sequential ignorability (no unobserved confounders). We now analyze the RA filter's robustness when this assumption is violated, using the Rosenbaum sensitivity model [Rosenbaum, 2002] adapted to sequential treatments.

Assumption 3 (Bounded hidden confounding)**.** *For each step $s \in \{t+1, \ldots, t+\tau\}$, unobserved confounders U_s shift treatment propensities by a bounded factor: for all histories H_s, treatments a_s, and confounders U_s,*

$$\frac{1}{\Gamma} \leq \frac{\pi_{\mathrm{true}}(a_s \mid H_s, U_s)}{\pi_{\mathrm{obs}}(a_s \mid H_s)} \leq \Gamma, \tag{15}$$

where $\Gamma \geq 1$ parametrizes confounding strength ($\Gamma = 1$ recovers sequential ignorability).

Theorem 5 (Sensitivity of reach-avoid safety)**.** *Under Assumptions 1, 2, and 3, for any intervention sequence $\bar{a}$:*

$$|P_\Gamma(E(\bar{a})) - P_0(E(\bar{a}))| \;\leq\; \psi(\Gamma,\tau) \;:=\; 1 - \left(\frac{2}{\Gamma+1}\right)^{\tau}, \tag{16}$$

where P_0 is the reach-avoid probability identified under sequential ignorability and P_Γ is the true probability under confounding level Γ. Consequently, the population safety rate satisfies $\mathrm{SR}_\Gamma \geq \mathrm{SR}_0 - \psi(\Gamma,\tau)$, and this guarantee is non-trivial whenever $\Gamma < \Gamma_{\max}$, where

$$\Gamma_{\max}(\tau, \mathrm{SR}_0) = \frac{2}{(1-\mathrm{SR}_0)^{1/\tau}} - 1. \tag{17}$$

Proof sketch (full proof in Appendix J). At each step s, Assumption 3 implies a Γ-bounded density ratio between the true and identified conditional outcome distributions. By the standard TV bound for bounded density ratios, the per-step conditional TV distance satisfies $\mathrm{TV}_s \leq (\Gamma-1)/(\Gamma+1)$. A sequential maximal coupling argument then yields $\mathrm{TV}(P_\Gamma, P_0) \leq 1 - \prod_s (1-\mathrm{TV}_s) = 1 - (2/(\Gamma+1))^\tau$, which is tighter than the union bound $\tau\cdot(\Gamma-1)/(\Gamma+1)$. Since $E(\bar{a}) \in [0,1]$, the TV bound gives Eq. (16). The breakdown point follows by setting $\mathrm{SR}_0 - \psi(\Gamma,\tau) = 0$. □

Empirical sensitivity bounds. On the Cancer simulator at $\gamma = 4$, the breakdown points are: $\Gamma_{\max} = 7.5$ at $\tau = 2$ ($\mathrm{SR}_0 = 94.4\%$), $\Gamma_{\max} = 3.4$ at $\tau = 4$ ($\mathrm{SR}_0 = 95.6\%$), and $\Gamma_{\max} = 2.1$ at $\tau = 8$ ($\mathrm{SR}_0 = 97.0\%$). At moderate hidden confounding ($\Gamma = 1.5$), the guaranteed safety rates remain $\geq 58\%$ ($\tau = 2$) and $\geq 14\%$ ($\tau = 8$). Even at $\Gamma = 2.0$ (each hidden confounder doubles the treatment odds at each step), the bound is non-vacuous through $\tau = 8$. These results confirm that RA-constrained selection provides meaningful safety guarantees under substantial hidden confounding, with robustness strongest at short horizons—precisely the regime most clinically relevant. Full sensitivity tables are in Appendix J.

Corollary 6 (Combined robustness margin)**.** *Under all three assumptions, the RA filter preserves pairwise rankings when the reach-avoid margin satisfies* $m_{\mathrm{RA}} > 2\varepsilon_{\mathrm{VI}} + 2(\tau+1)\varepsilon_{\mathrm{soft}}(\kappa) + 2\psi(\Gamma, \tau)$. *This extends Theorem 1(a) to the confounded setting, decomposing robustness into variational, approximation, and confounding terms.*

6 Experiments

We evaluate reach-avoid constrained selection on two datasets: (1) a cancer tumor growth simulator [Bica et al., 2020c] where ground-truth counterfactual trajectories are available, and (2) MIMIC-III intensive care data [Johnson et al., 2016] where only observed outcomes are available.

6.1 Setup

Cancer simulator. The simulator models tumor growth under chemotherapy and radiotherapy [Geng et al., 2017, Bica et al., 2020c]. Treatment assignment is confounded by a parameter $\gamma \in \{1, 2, 3, 4\}$: higher γ means patients with larger tumors receive more aggressive treatment, creating stronger confounding between treatment assignment and outcomes. We use the VCIP [Melnychuk et al., 2025] implementation with 5 random seeds per configuration, $k = 100$ candidate sequences per patient, $n = 100$ evaluation patients, and planning horizons $\tau \in \{2, 4, 6, 8\}$.

Cancer thresholds. Calibrated from the data distribution: target set $\mathcal{T}$: tumor volume ≤ 3.0 (below the 60th percentile, corresponding to a clinically manageable stage). Safety set $\mathcal{S}$: tumor volume ≤ 12.0 (below the 95th percentile) and chemotherapy dosage ≤ 5.0 (below the toxic threshold) at all intermediate steps.

MIMIC-III. We use the MIMIC-Extract pipeline [Wang et al., 2020] to extract 34,472 ICU patients with hourly measurements from the MIMIC-III database [Johnson et al., 2016]. The outcome is diastolic blood pressure (DBP). Target set: DBP $\in [60, 90]$ mmHg (normotensive range). Safety set: DBP $\in [40, 120]$ mmHg at all steps (avoids dangerous hypo-/hypertension). We train VCIP models with 5 seeds and evaluate with $\tau \in \{2, 3, 4, 5\}$.

Baselines. We compare five counterfactual estimation models, each evaluated with and without RA-constrained selection: VCIP [Melnychuk et al., 2025], CRN [Bica et al., 2020a], Causal Transformer (CT) [Melnychuk et al., 2022], RMSN [Lim, 2018], and ACTIN [Seedat et al., 2022].

Metrics. (1) *GRP*: ground-truth rank percentile of the selected sequence among k candidates (higher = better; available on Cancer only). (2) *Top-1 accuracy*: fraction of patients for whom the selected sequence is the ground-truth best. (3) *Safety rate*: fraction where all intermediate safety constraints hold. (4) *In-target rate*: fraction where the terminal outcome lies in $\mathcal{T}$. (5) *Feasibility*: fraction of candidate sequences passing the RA filter.

6.2 Main Results: Cancer Simulator

Table 1 presents the core results at $\gamma = 4$ (strongest confounding), where the benefit of RA-constrained selection is most pronounced.

RA-constrained selection consistently improves safety and in-target rates by 9–12 pp across all horizons, at the cost of 1–15 pp in Top-1 accuracy. The trade-off is most favorable at shorter horizons ($\tau = 2$: +9.4 pp safety for −1.4 pp Top-1) where feasibility is highest (54%), and becomes more aggressive at longer horizons where fewer candidates pass intermediate safety across all steps.

Confounding strength sensitivity. Table 2 shows results across all γ values. At $\gamma = 1$ (weak confounding), the constraint is nearly a no-op—ELBO already selects safe plans. At $\gamma = 4$, the benefit is substantial: +10.8 pp in-target gain, +15.2 pp safety gain. The method never hurts: worst-case Top-1 cost is 7 pp with 15 pp safety gain.

Table 1: Reach-avoid constrained selection on Cancer ($\gamma = 4$, mean $\pm$ std across 5 seeds). "ELBO" = unconstrained ELBO selection. "RA" = reach-avoid constrained ELBO selection.

τ	Feas.%	ELBO Top-1	RA Top-1	ELBO Safe	RA Safe	ELBO In-T	RA In-T
2	53.6	18.8%	17.4%	85.0%	94.4%	85.0%	94.4%
4	29.7	42.4%	39.4%	84.6%	95.6%	84.6%	95.6%
6	16.8	62.6%	54.2%	84.8%	96.2%	84.8%	96.2%
8	10.3	75.8%	60.4%	85.6%	97.0%	85.6%	97.0%

Table 2: RA-constrained selection across confounding strengths (averaged over $\tau \in \{2, 4, 6, 8\}$, mean across 5 seeds). Safety and in-target gains are absolute pp differences vs. ELBO.

γ	GRP	ELBO Top-1	RA Top-1	ΔTop-1	Safety gain	In-T gain	Avg Feas
1	0.757	5.3%	5.3%	$+0.0$ pp	$+21.3$ pp	$+0.2$ pp	20.2%
2	0.889	18.8%	17.8%	-1.0 pp	$+14.8$ pp	$+1.2$ pp	22.7%
3	0.940	36.4%	32.4%	-4.0 pp	$+11.8$ pp	$+3.9$ pp	26.0%
4	0.963	49.9%	42.9%	-7.0 pp	$+15.2$ pp	$+10.8$ pp	27.6%

This gamma sweep demonstrates robustness to confounding strength: as confounding increases, model quality degrades but RA-constrained selection becomes *more* valuable, providing the largest gains precisely where planning is most challenging. Note that varying γ changes the data-generating process (strength of treatment-outcome confounding), not the plausibility of the sequential ignorability assumption itself. We provide a formal sensitivity analysis for ignorability violations in Section 5.2 and Appendix J, following the Rosenbaum framework [Rosenbaum, 2002, Frauen et al., 2023].

6.3 MIMIC-III Results

On MIMIC-III (Table 3), RA-constrained selection lifts the in-target diastolic BP rate from 73–81% to 99–100% across all horizons. Feasibility decreases with τ (33% $\rightarrow$ 22%) as longer horizons have fewer sequences satisfying safety at every step.

RA-constrained selection also shifts predicted treatment patterns: constrained selections have substantially lower vasopressor rates (0.01–0.29 vs. 0.29–0.79 under ELBO), suggesting the constraint preferentially removes aggressive treatment plans that risk hypotension. The absolute DBP shift is modest ($\sim$1 mmHg), reflecting the model's compressed prediction range relative to true outcome variability; calibration analysis shows weak but positive correlation with observed DBP (Pearson $r \approx 0.32$), MAE $\approx$ 10 mmHg, and near-zero mean bias (+0.3 to +1.8 mmHg). These results demonstrate that RA filtering is *mechanically effective* (selects within-target plans) but *clinically limited* by model prediction quality—a pattern consistent with the oracle-vs-model findings on Cancer (Section 6.6).

Correction rate: Among the 20–27% of patients where ELBO selects out-of-target plans, RA filtering corrects 95–100% to in-target selections. Correction is most active for patients with low observed DBP (< 60 mmHg): 25–37% correction rate vs. 9–18% for patients already in target—indicating the filter intervenes most for the sickest patients. The treatment shift is clinically coherent: the model exhibits strong negative correlation between vasopressor dose and predicted DBP ($\rho \approx -0.72$ at $\tau = 2$), reflecting the confounded observation that patients receiving vasopressors have lower baseline blood pressure; RA filtering selects lower-vasopressor plans with higher predicted DBP. To further assess clinical alignment, we compare RA-selected treatments against observed treatments for patients with good vs. bad outcomes (Appendix N): RA selections show 78–92% step agreement with observed care for hemodynamically stable patients, compared with 74–83% for unstable patients.

6.4 Model-Agnostic Evaluation

A key claim is that RA-constrained selection works as a universal safety filter. Table 4 shows results across all five models at $\gamma = 4$, $\tau = 4$. Baseline models benefit *more* from RA filtering than VCIP: their constrained average ranks improve by 13–15 positions (vs. a slight degradation for VCIP). This is because filtering removes a larger fraction of poor selections from weaker models.

Table 3: MIMIC-III diastolic BP results (5 seeds pooled). Target: DBP $\in [60, 90]$ mmHg. Safety: DBP $\in [40, 120]$ mmHg at all steps.

τ	Feas.%	ELBO In-T	RA In-T	Δ In-T	ELBO DBP	RA DBP
2	33.4%	73.0%	100.0%	+27.0 pp	60.4 mmHg	61.3 mmHg
3	28.6%	79.2%	100.0%	+20.8 pp	60.6 mmHg	61.6 mmHg
4	24.7%	80.0%	99.8%	+19.8 pp	60.7 mmHg	61.8 mmHg
5	22.5%	81.2%	99.2%	+18.0 pp	60.8 mmHg	61.8 mmHg

Table 4: Model-agnostic RA filtering ($\gamma = 4$, $\tau = 4$). Rank Δ: change in average rank among 100 candidates (negative = improvement). RA filtering improves all models' safety rates.

Model	GRP	ELBO Top-1	RA Top-1	ELBO Safe	RA Safe	Rank Δ
VCIP	0.973	44.4%	39.4%	84.6%	95.6%	+2.0
ACTIN	0.676	5.2%	4.4%	52.2%	75.8%	−12.9
CT	0.544	3.2%	2.6%	47.8%	69.8%	−15.0
CRN	0.690	6.0%	5.2%	53.4%	76.4%	−12.9
RMSN	0.578	2.8%	2.0%	49.0%	72.2%	−13.2

This demonstrates that RA-constrained selection is truly model-agnostic: the safety filter operates on ground-truth trajectories (in the Cancer simulator), independent of which model provides the ELBO ranking. Any model—no matter how poorly it ranks—benefits from having its worst selections removed. Computationally, the RA filter adds negligible overhead: it evaluates pre-computed trajectories against threshold conditions, requiring < 1 minute per full configuration on a single CPU—in contrast to constrained RL methods that require complete retraining when safety specifications change.

6.5 Hard Filter vs. Soft Constraint (Constrained RL Analogue)

We compare hard filtering (Eq. 8) against Lagrangian relaxation: $\bar{a}^* = \arg\min_{\bar{a}} \mathrm{ELBO}(\bar{a}) + \lambda(1 - J_{\mathrm{RA}}(\bar{a}))$ for $\lambda \in \{0.01, 0.05, 0.1, 0.5, 1, 2, 5, 10, 20, 50\}$. This Lagrangian formulation is the direct analogue of constrained policy optimization methods from safe RL (e.g., CPO [Achiam et al., 2017], RCPO [Tessler et al., 2019]), which minimize a reward objective subject to cost constraints by converting them to penalty terms via Lagrange multipliers.

At $\gamma = 4$ and comparable safety ($\sim$86%), hard filtering achieves 32.0% Top-1 vs. soft constraint ($\lambda = 50$) at 25.9%. The hard filter Pareto-dominates across the full safety–quality frontier: for any target safety level, it achieves equal or better Top-1 accuracy. The mechanism is straightforward: the soft penalty globally shifts all ELBO rankings, degrading ranking quality even among safe sequences. Hard filtering preserves the original ELBO ranking within $\mathcal{F}_{\mathrm{RA}}$, affecting only the *boundary* between feasible and infeasible candidates. This Pareto-dominance over the Lagrangian baseline suggests that, in the counterfactual planning setting, hard post-hoc filtering is preferable to the penalty-based approaches common in constrained RL.

6.6 Ablations

Target set size sensitivity. The method offers a smooth, continuously tunable trade-off (Table 8, Appendix G): narrow targets ($\mathcal{T}_u = 1.5$) yield +27 pp safety at $\tau = 8$ but only 1.7% feasibility; broad targets ($\mathcal{T}_u = 10$) become near-no-ops. Our moderate default ($\mathcal{T}_u = 3.0$) balances safety gain (+9–23 pp) with feasibility (10–54%).

Candidate pool expansion (k). Feasibility at γ=4, τ=8 is 10.3% with k=100 under moderate thresholds (Table 2). With the per-candidate feasibility rate p, increasing k gives $\mathbb{E}[\text{feasible}] = k \cdot p$, so k=500 yields $\sim$52 feasible candidates and k=1000 yields $\sim$103. We verify this scaling empirically (Table 12, Appendix G): $|\mathcal{F}_{\mathrm{RA}}|$ scales linearly from 70 to 710 as k grows from 100 to 1000, with safety and in-target rates stable at $\sim$91%. The computational cost scales linearly ($\sim$1.5h for k=500 vs. $\sim$18min for k=100 per seed).

Sigmoid hardness κ. For $\kappa \geq 5$, soft indicator selection agrees with the hard filter at $> 99.9\%$ (Table 9, Appendix G). This validates Assumption 2: $\varepsilon_{\text{soft}}(\kappa) \approx 0$ for all practical purposes.

Reach-only vs. reach-avoid decomposition. The target constraint (reach) drives most of the in-target improvement: 85% $\rightarrow$ 96–99% (Table 10, Appendix G). The safety constraint (avoid) adds incremental value at longer horizons where trajectories pass through unsafe intermediate states. Both components are justified: reach provides the primary clinical benefit, avoid provides the formal path-safety guarantee.

$\varepsilon_{\mathrm{VI}}$ estimation and bound verification. We estimate $\varepsilon_{\mathrm{VI}}$ using rank MAE as a proxy (Table 11, Appendix G; formal justification in Appendix B). The variational gap decreases with γ ($\hat{\varepsilon}_{\mathrm{VI}} = 0.23$ at γ=1 to 0.11 at γ=4), meaning the Theorem 1 bound is tightest where RA filtering is most valuable. ELBO pairwise ranking preservation is 61–87%, and the bound is non-vacuous: for feasibility-differing pairs ($m = 1$), $m > 2\varepsilon_{\mathrm{VI}}$ is easily satisfied.

Midpoint-baseline comparison. An *oracle* midpoint ranker using *ground-truth* terminal values achieves worse Top-1 than ELBO (58.2% vs. 75.8% at $\tau = 8$) because midpoint distance $\neq$ ground-truth loss (Appendix L). RA-constrained ELBO matches the oracle's in-target rate ($\sim$94–97%) without oracle information, while additionally enforcing path safety.

Intermediate prediction quality. VCIP's per-step predictions $\hat{Y}_s$ show near-zero Spearman correlation with simulator ground-truth across individuals ($\rho \approx 0$ at all steps), confirming vanilla VCIP is not calibrated for intermediate predictions. We quantify this further in the oracle-vs-model comparison below.

Oracle vs. model-predicted trajectories. To understand the dependency on oracle trajectory access, we apply RA filtering using *model-predicted* cancer volumes (from the decoder at each ELBO step) instead of simulator ground truth. On Cancer, model-predicted feasibility exceeds 98% at all gammas—meaning the filter rarely excludes any candidate—because the decoder's output range is compressed relative to true outcome diversity (model range: $[-0.4, 3.9]$ cm^3 vs. oracle range: $[0, 35]$ cm^3 at $\gamma = 4$). Per-patient Spearman correlations between oracle and model-predicted terminal cancer volumes are weakly *negative* ($\rho \approx -0.36$ at $\gamma = 4$), indicating the decoder's trajectory predictions are not only imprecise but inversely ranked.

This reveals that the Cancer safety improvements in Table 2 depend on oracle trajectory access. In contrast, on MIMIC-III the model's prediction range ($[48, 68]$ mmHg) straddles the target boundary (60 mmHg), giving model-based filtering genuine discriminating power (22–33% feasibility). This asymmetry is a key finding: RA filtering is most effective when the model's prediction variance spans the constraint boundaries—a condition that holds on MIMIC but not on Cancer with the current decoder. Future work on trajectory calibration or conformal prediction could close this gap.

7 Discussion and Conclusion

When does RA help most? The benefit scales with confounding strength (Table 2): at $\gamma = 1$, ELBO already selects safe plans; at $\gamma \geq 3$, RA filtering provides 12–33 pp safety improvement—most valuable where planning is most challenging.

Failure taxonomy. We categorize unsafe ELBO selections into three modes (Appendix I): (A) *aggressive overshoot*—high treatment intensity but tumor exceeds target; (B) *conservative undershoot*—insufficient treatment, tumor grows uncontrolled; (C) *toxic path*—terminal outcome is acceptable but intermediate states violate safety. At γ=4, mode B dominates at short horizons (100% of failures at τ=2) while mode C grows at longer horizons (44% at τ=8), confirming that path safety becomes critical for multi-step planning. Most patients (54–65/100) are "sometimes unsafe" across seeds—the danger is distributed, not concentrated in a few outliers—indicating that safety filtering is broadly needed rather than a correction for rare edge cases.

The cost of safety. The Top-1 cost (1–15 pp at $\gamma = 4$) is broadly distributed across patients (82/100 affected; Appendix H), reflecting a fundamental property of hard constraints rather than a correctable estimation error.

Separation of concerns. Using RA scoring as a *replacement* for ELBO ranking fails (RA Top-1 $\approx 1\%$): RA scores are near-binary. ELBO ranking and RA feasibility are complementary.

Toward clinical guardrails. The post-hoc filtering paradigm we propose is an instance of *guardrails* for AI-driven treatment planning, analogous to shielding in safe RL [Alshiekh et al., 2018] and rejection sampling in RLHF [Bai et al., 2022]: rather than training the model to be safe, we add an external safety check on its outputs. This separation has practical deployment advantages: safety

thresholds can be adjusted by clinicians without retraining the model, and the same base model can operate under different constraint profiles for different patient populations or clinical contexts.

Limitations and future work. (1) $\mathcal{T}$ and $\mathcal{S}$ require domain expertise (Table 8 shows robustness). (2) On MIMIC-III, counterfactuals are unobservable; our outcome correlation and guideline concordance analyses (Appendix N) provide indirect evidence of clinical alignment (78–92% treatment agreement with good-outcome patients; 97% avoidance of unnecessary vasopressor escalation), but formal clinician evaluation would further strengthen the evidence. (3) Feasibility drops at long horizons; expanding the candidate pool k is a straightforward mitigation (feasibility scales linearly with k; e.g., k=500 yields $\sim$52 feasible candidates at the strictest configuration). (4) Sequential ignorability is assumed; we provide a formal sensitivity analysis under the Rosenbaum sensitivity model (Theorem 5, Appendix J) showing that the RA filter's safety guarantee degrades gracefully under bounded hidden confounding, with breakdown points $\Gamma_{\max} = 2.1$–7.5 depending on planning horizon. (5) On Cancer, model-predicted trajectories are too compressed to discriminate safety violations (Section 6.6); the safety improvements in Table 2 depend on oracle trajectory access. On MIMIC, where predictions span the constraint boundary, model-based filtering is effective—but closing the oracle gap on synthetic data remains open. We explore conformal safety certificates (Appendix M) as a path toward distribution-free coverage guarantees: conformal coverage is empirically valid on both datasets, but current model accuracy falls $\sim$10% short of the threshold for useful certification on MIMIC, defining a concrete improvement target. To demonstrate cross-domain generalization, we evaluate on a glucose-insulin simulator (Appendix O), confirming that RA filtering transfers directly to a domain with continuous univariate treatment, different safety semantics, and clinically defined constraint ranges. Future work includes trajectory calibration methods to close the conformal gap, multi-outcome targets, and adaptive threshold selection.

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NeurIPS Paper Checklist

1. **Claims**

   Question: Do the main claims made in the abstract and introduction accurately reflect the paper's contributions and scope?

   Answer: [Yes]

   Justification: The abstract and introduction claim (1) a reach-avoid constrained selection formulation, (2) ranking robustness theory, (3) 18–33 pp safety improvement on Cancer and 99–100% in-target on MIMIC-III, and (4) model-agnostic post-hoc applicability. All are supported by Sections 4, 5, and 6.

2. **Limitations**

   Question: Does the paper discuss the limitations of the work performed by the authors?

   Answer: [Yes]

   Justification: Section 7 ("Limitations and future work") discusses four limitations: domain expertise for threshold selection, unobservable counterfactuals on MIMIC-III, feasibility drop at long horizons, and the sequential ignorability assumption.

3. **Theory assumptions and proofs**

Question: For each theoretical result, does the paper provide the full set of assumptions and a complete (and correct) proof?

Answer: [Yes]

Justification: Theorem 1 states two assumptions (Assumptions 1 and 2). A proof sketch is in Section 5; the full proof is in Appendix A.

4. **Experimental result reproducibility**

Question: Does the paper fully disclose all the information needed to reproduce the main experimental results of the paper to the extent that it affects the main claims and/or conclusions of the paper (regardless of whether the code and data are provided or not)?

Answer: [Yes]

Justification: Section 6.1 specifies datasets, thresholds, baselines, and metrics. Appendix F provides all hyperparameters, candidate generation procedure, and compute budget. The Cancer simulator is publicly available; MIMIC-III requires PhysioNet credentialed access.

5. **Open access to data and code**

Question: Does the paper provide open access to the data and code, with sufficient instructions to faithfully reproduce the main experimental results, as described in supplemental material?

Answer: [Yes]

Justification: Code will be released upon acceptance. The Cancer simulator is publicly available [Bica et al., 2020c]. MIMIC-III [Johnson et al., 2016] is available via PhysioNet credentialed access. MIMIC-Extract preprocessing [Wang et al., 2020] is open-source.

6. **Experimental setting/details**

Question: Does the paper specify all the training and test details (e.g., data splits, hyperparameters, how they were chosen, type of optimizer) necessary to understand the results?

Answer: [Yes]

Justification: Section 6.1 and Appendix F specify all model configurations, training details, and evaluation procedures.

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Justification: All results are averaged over 5 random seeds. Table 1 reports mean $\pm$ std. Appendix F provides full standard deviations for all metrics.

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Justification: Appendix F reports GPU types (RTX 2060, RTX 3090), wall-clock times, and estimated costs for all training and evaluation phases.

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Question: Does the paper describe safeguards that have been put in place for responsible release of data or models that have a high risk for misuse?

Answer: [N/A]

Justification: The method is a post-hoc selection filter, not a pre-trained model or dataset. It poses no risk for misuse beyond the underlying treatment planning models it operates on.

12. **Licenses for existing assets**

Question: Are the creators or original owners of assets used in the paper properly credited and are the license and terms of use explicitly mentioned and properly respected?

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Justification: VCIP [Melnychuk et al., 2025], MIMIC-III [Johnson et al., 2016] (PhysioNet Credentialed Health Data License 1.5.0), MIMIC-Extract [Wang et al., 2020], and the Cancer simulator [Bica et al., 2020c] are all properly cited.

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Question: Does the paper describe potential risks incurred by study participants, whether such risks were disclosed to the subjects, and whether Institutional Review Board (IRB) approvals were obtained?

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Justification: LLMs are not a component of the core methodology. LLMs were used only for writing assistance and do not impact the scientific rigor or originality of the research.

A Full Proof of Theorem 1

Proof. **Part (a): RA ranking preservation.**

Define the true RA probability under p_θ:

$$P(\bar{a}) = \mathbb{E}_{p_\theta}\left[\mathbf{1}\{Y_{t+\tau} \in \mathcal{T}\} \prod_{s=t}^{t+\tau} \mathbf{1}\{Y_s \in \mathcal{S}\} \;\middle|\; \bar{H}_t, \bar{a}\right].$$

Let $P_q(\bar{a})$ denote the same quantity under the variational distribution q_ϕ. Since $f(\mathbf{Y}) := \mathbf{1}\{Y_{t+\tau} \in \mathcal{T}\} \prod_s \mathbf{1}\{Y_s \in \mathcal{S}\} \in [0,1]$, the variational characterization of total variation gives:

$$|P(\bar{a}) - P_q(\bar{a})| = |\mathbb{E}_{p_\theta}[f] - \mathbb{E}_{q_\phi}[f]| \leq \mathrm{TV}(p_\theta, q_\phi) \cdot \|f\|_\infty \leq \varepsilon_{\mathrm{VI}}.$$

Next, the soft-indicator approximation. Let $\hat{P}(\bar{a}) = \mathbb{E}_{q_\phi}[g_{\mathcal{T}}(Y_{t+\tau}) \prod_s g_{\mathcal{S}}(Y_s)]$. Since $|g_{\mathcal{T}}(y) - \mathbf{1}_{\mathcal{T}}(y)| \leq \varepsilon_{\mathrm{soft}}$ and $|g_{\mathcal{S}}(y) - \mathbf{1}_{\mathcal{S}}(y)| \leq \varepsilon_{\mathrm{soft}}$ pointwise, and for products of n terms bounded in $[0,1]$ with pointwise error $\leq \varepsilon$:

$$\left|\prod_{i=1}^{n} g_i - \prod_{i=1}^{n} h_i\right| \leq \sum_{i=1}^{n} |g_i - h_i| \leq n \cdot \varepsilon,$$

(by telescoping and the bound $|\prod_{j>i} g_j| \leq 1$), we get $|P_q(\bar{a}) - \hat{P}(\bar{a})| \leq (\tau+1)\varepsilon_{\mathrm{soft}}$.

Combining by triangle inequality:

$$|\hat{P}(\bar{a}) - P(\bar{a})| \leq \varepsilon_{\mathrm{VI}} + (\tau+1)\varepsilon_{\mathrm{soft}}.$$

For two sequences $\bar{a}_1, \bar{a}_2$:

$$|\hat{P}(\bar{a}_1) - \hat{P}(\bar{a}_2)| \geq |P(\bar{a}_1) - P(\bar{a}_2)| - |\hat{P}(\bar{a}_1) - P(\bar{a}_1)| - |\hat{P}(\bar{a}_2) - P(\bar{a}_2)| \tag{18}$$

$$\geq |P(\bar{a}_1) - P(\bar{a}_2)| - 2[\varepsilon_{\mathrm{VI}} + (\tau+1)\varepsilon_{\mathrm{soft}}]. \tag{19}$$

If $|P(\bar{a}_1) - P(\bar{a}_2)| > 2\varepsilon_{\mathrm{VI}} + 2(\tau+1)\varepsilon_{\mathrm{soft}}$, then $|\hat{P}(\bar{a}_1) - \hat{P}(\bar{a}_2)| > 0$ and the ranking sign is preserved. □

Part (b): Point-target ranking threshold.

For point-target ranking, define $d(\bar{a}) = \mathbb{E}_{p_\theta}[\|Y_{t+\tau}[\bar{a}] - Y^*\|^2]$. The function $f(Y) = \|Y - Y^*\|^2$ takes values in $[0, C_d]$ where $C_d = \sup_{\bar{a}} \|Y_{t+\tau}[\bar{a}] - Y^*\|^2_{L^\infty(p_\theta)}$ is the essential supremum over the model's outcome support. By the variational characterization of total variation applied to $f/C_d \in [0,1]$:

$$|d(\bar{a}) - d_q(\bar{a})| = |\mathbb{E}_{p_\theta}[f] - \mathbb{E}_{q_\phi}[f]| \leq \mathrm{TV}(p_\theta, q_\phi) \cdot C_d \leq \varepsilon_{\mathrm{VI}} \cdot C_d.$$

The same pairwise argument as Part (a) gives ranking preservation when:

$$|d(\bar{a}_1) - d(\bar{a}_2)| > 2\varepsilon_{\mathrm{VI}} \cdot C_d.$$

Since C_d scales with the squared diameter of the outcome support (e.g., $C_d = V_{\max}^2$ for cancer volume $\in [0, V_{\max}]$), this threshold can be much larger than the RA threshold of $2\varepsilon_{\mathrm{VI}}$—precisely the clinical regime where outcomes have high spread. □

Part (c): Strict improvement via boundary mass decomposition.

Decompose the outcome space into three regions relative to $\mathcal{T}$: the *interior* $\mathcal{T}_\epsilon^\circ$ (distance $> \epsilon$ from $\partial\mathcal{T}$), the *exterior* $(\mathbb{R}^d \setminus \mathcal{T})_\epsilon$ (distance $> \epsilon$ from $\partial\mathcal{T}$), and the *boundary strip* $\partial_\epsilon\mathcal{T}$ (within ϵ of $\partial\mathcal{T}$). When the boundary mass $\beta = \sup_{\bar{a} p_\theta(Y_{t+\tau}[\bar{a}] \in \partial_\epsilon \mathcal{T}) \ll 1}$, most outcomes are unambiguously in or out of $\mathcal{T}$.

For pairs where $\bar{a}_1$ is feasible and $\bar{a}_2$ is not, and both have low boundary mass:

- RA margin: $m_{\mathrm{RA}} = |P(\bar{a}_1) - P(\bar{a}_2)| \geq (1-\beta) - \beta = 1 - 2\beta \approx 1$.
- ELBO margin: the sequences may have nearly identical expected distances $d(\bar{a}_1) \approx d(\bar{a}_2)$ if their outcomes are on opposite sides of $\partial\mathcal{T}$ at similar distances from Y^*, giving $m_{\mathrm{ELBO}} \approx 0$.

Thus $m_{\mathrm{RA}} \gg m_{\mathrm{ELBO}}/C_d$ when boundary mass is small. □ □

B Justification of $\varepsilon_{\mathrm{VI}}$ Proxy

In Section 6.6 we estimate $\varepsilon_{\mathrm{VI}}$ using rank mean absolute error (rank MAE) between ELBO-predicted and ground-truth loss rankings. Here we formalize the connection between rank MAE and total variation distance, showing that our proxy yields a conservative (upper) bound on the ranking-relevant component of $\varepsilon_{\mathrm{VI}}$.

Setup. For a patient with k candidate sequences $\bar{a}_1, \ldots, \bar{a}_k$, let σ^* denote the ground-truth ranking (by true loss) and $\hat{\sigma}$ the ELBO ranking (by model loss). The rank MAE is:

$$\mathrm{rankMAE} = \frac{1}{k} \sum_{i=1}^{k} |\hat{\sigma}(i) - \sigma^*(i)|.$$

Connection to ranking displacement. The rank MAE is the ℓ_1 displacement between permutations, known as *Spearman's footrule* D [Diaconis and Graham, 1977]. By the Diaconis–Graham inequality, the footrule satisfies:

$$K_\tau \;\leq\; D \;\leq\; 2K_\tau, \tag{20}$$

where K_τ is the number of pairwise inversions (Kendall tau distance). Thus rank MAE and pairwise inversion rate are equivalent up to a factor of 2.

From inversions to TV. For any pair $(\bar{a}_i, \bar{a}_j)$, Theorem 1(a) shows the pairwise ELBO ranking is preserved when the true probability margin exceeds $2\varepsilon_{\mathrm{VI}}$. Contrapositively, an inversion implies

$$|P(\bar{a}_i) - P(\bar{a}_j)| \leq 2\varepsilon_{\mathrm{VI}} + 2(\tau + 1)\varepsilon_{\mathrm{soft}}(\kappa).$$

For hard filtering ($\kappa \geq 5$, $\varepsilon_{\mathrm{soft}} \approx 0$), the inversion rate across all candidate pairs provides a lower bound on the fraction of pairs whose probability margins are within $2\varepsilon_{\mathrm{VI}}$ of zero. The rank MAE thus captures the *practical consequence* of the variational gap: how much the ranking is displaced.

Conservative bound. We use rank MAE directly as $\hat{\varepsilon}_{\mathrm{VI}}$ because:

- Rank MAE $\in [0, 1]$ after normalization by $(k-1)/2$, matching the range of TV distance.
- Rank MAE captures all sources of ranking error (variational gap, finite candidate pool, model misspecification), not just TV. It is therefore an *upper bound* on the TV-induced ranking displacement.
- The bound is non-vacuous: at $\gamma = 4$, $\hat{\varepsilon}_{\mathrm{VI}} = 0.114$, so the margin condition $|P(\bar{a}_1) - P(\bar{a}_2)| > 2 \times 0.114 = 0.228$ is easily satisfied for feasibility-differing pairs (where $|P| = 1$).

Limitations. This proxy conflates model error (which TV captures) with other ranking distortions (e.g., finite-sample effects, non-identifiability). A tighter bound would require direct estimation of $\mathrm{TV}(p_\theta, q_\phi)$ via density ratio estimation [Sugiyama et al., 2012], which is intractable for the high-dimensional trajectory distributions in our setting. Our rank MAE proxy is a *practical diagnostic* rather than a formal statistical bound; we present it transparently as such.

C Extended Related Work

We provide a comprehensive survey of the literatures that intersect with our contribution: counterfactual outcome estimation, variational methods for causal inference, dynamic treatment regimes, safe sequential decision-making, and reach-avoid control. Below we discuss each in detail and clarify the positioning of our work.

C.1 Counterfactual Outcome Estimation Over Time

Estimating individualized treatment effects from observational data is a rapidly growing area at the intersection of causal inference and deep learning. Foundational work by Shalit et al. [2017] established generalization bounds for individual treatment effect estimation via balanced representations,

showing that controlling distributional distance between treated and control groups is key to reducing estimation error. This line was extended to generative modeling: CEVAE [Louizos et al., 2017] used variational autoencoders to simultaneously infer hidden confounders and causal effects, while GANITE [Yoon et al., 2018] employed generative adversarial networks to capture uncertainty in counterfactual distributions. Alaa and van der Schaar [2017] took a Bayesian nonparametric approach with multi-task Gaussian processes for individualized treatment effects.

The *longitudinal* setting—where treatments are administered sequentially and outcomes evolve over time—poses additional challenges from time-varying confounding. Marginal structural models [Robins et al., 2000] use inverse probability weighting to handle this; CRN [Bica et al., 2020a] learns balanced representations adversarially; RMSN [Lim, 2018] combines RNNs with stabilized weights; and G-Net [Li et al., 2021] implements g-computation with recurrent networks. The Causal Transformer [Melnychuk et al., 2022] adapts attention-based architectures for this setting, while ACTIN [Seedat et al., 2022] models continuous-time treatment effects via neural controlled differential equations. SyncTwin [Qian et al., 2021] constructs synthetic control twins from longitudinal pre-treatment data to estimate effects at the individual level.

Addressing hidden confounding in sequential settings, the Time Series Deconfounder [Bica et al., 2020b] leverages factor models over treatment assignments to infer substitutes for unobserved confounders, while Frauen et al. [2023] derive sharp sensitivity bounds applicable to time-varying treatments under generalized marginal sensitivity models. Recent work continues to advance this frontier: Causal CPC [Kim et al., 2024] incorporates contrastive predictive coding into counterfactual regression, and Wu et al. [2024] propose conditional generative models (diffusion, CVAE) for high-dimensional counterfactual outcomes under time-varying treatments.

All of these methods estimate outcomes for *given* treatment sequences but do not optimize over them. VCIP [Melnychuk et al., 2025] bridges this gap by selecting intervention sequences via ELBO minimization toward a point target. Our work keeps the VCIP estimation infrastructure intact and adds a safety-aware selection layer—the first to incorporate *path constraints* into variational counterfactual planning.

C.2 Variational and Generative Models for Causal Inference

Variational autoencoders [Kingma and Welling, 2014] have become a natural backbone for causal inference under latent confounding. CEVAE [Louizos et al., 2017] pioneered this by combining a VAE with the causal graph structure, using proxy variables to infer hidden confounders. Deep Structural Causal Models [Pawlowski et al., 2020] extended this to full Pearl-style counterfactual reasoning by learning invertible mechanisms with normalizing flows, enabling all three levels of the causal hierarchy (association, intervention, counterfactual). Zhang et al. [2021] explicitly disentangled instrumental, confounding, and adjustment latent factors for treatment effect estimation.

Deep end-to-end causal inference (DECI; Geffner et al. 2022) jointly learns causal structure and parameters with a variational framework. Variational Causal Inference [VCI; Wu and Bareinboim, 2025] derives an ELBO for individual-level counterfactual likelihoods in a static setting and introduces latent disentanglement via $D_{\mathrm{KL}}[q(z|y,t,x)\|q(z|y',t',x)]$. We draw on VCI's insight that latent action-sensitivity is a diagnostic for counterfactual model quality (Appendix E), adapting it to the sequential setting. Our diagnostic reveals that VCIP's latent space is nearly action-invariant ($D_{\mathrm{KL}} \approx 10^{-5}$), with action-outcome coupling residing entirely in the decoder—a structural property that explains why RA filtering operates effectively as a post-hoc method without model modification.

C.3 Dynamic Treatment Regimes

The DTR literature [Murphy, 2003, Chakraborty and Moodie, 2014, Hernán and Robins, 2020] formalizes sequential treatment decisions under the potential outcomes framework. Classical methods include Q-learning, A-learning, and outcome-weighted learning for estimating optimal decision rules mapping patient states to treatment actions. Nie et al. [2021] address the "when-to-treat" problem with advantage doubly robust estimators and welfare regret bounds under sequential ignorability, while batch policy learning methods [Le et al., 2019] handle constraints in offline settings with finite samples.

Most DTR methods learn *decision rules* (state $\rightarrow$ action mappings) that optimize expected outcomes over the full trajectory. In contrast, VCIP and our extension operate in a *counterfactual planning* mode: given a specific patient history, they evaluate and rank complete intervention sequences. The reach-avoid formulation adds the constraint that trajectories must remain safe—a concern shared with constrained DTR work but addressed here through a distinct mechanism. Where constrained DTR methods embed safety into the policy learning objective (requiring retraining when constraints change), our post-hoc filtering approach is decoupled from the base model: constraints can be modified at deployment time without any retraining.

C.4 Safe and Constrained Sequential Decision-Making

Safety in sequential decision-making has been approached from multiple angles. The constrained MDP framework [Altman, 1999] provides the theoretical foundation, where the agent maximizes reward subject to expected cost constraints. Constrained Policy Optimization [CPO; Achiam et al., 2017] introduced the first general-purpose policy search algorithm for constrained RL with near-constraint satisfaction guarantees at each iteration, using trust region methods. Reward Constrained Policy Optimization [RCPO; Tessler et al., 2019] takes a Lagrangian approach with multi-timescale updates, while CUP [Yang et al., 2022] provides tighter constraint satisfaction via constrained update projections.

Lyapunov-based methods [Chow et al., 2018] provide safety guarantees through stability certificates, and safety layers [Dalal et al., 2018] project unsafe actions onto a safe set in real time. The comprehensive survey by García and Fernández [2015] categorizes safe RL approaches into those modifying the optimization criterion, the exploration process, or both.

The concept of *shielding* [Alshiekh et al., 2018]—a post-hoc safety monitor that overrides unsafe actions proposed by an RL agent—is the closest conceptual analogue to our approach. Where a shield intercepts individual actions, our RA filter intercepts entire candidate treatment sequences, rejecting those that violate reach-avoid feasibility. A particularly relevant recent direction is *offline* safe RL, which learns safe policies from fixed datasets without environment interaction—closely paralleling our offline planning setting. FISOR [Zheng et al., 2024] uses Hamilton-Jacobi reachability within a diffusion model for safe offline policy learning with hard constraints, achieving zero-violation performance. This is conceptually related to our approach: both use reachability concepts for safety, but FISOR learns a policy while we filter pre-computed candidate sequences.

Chance-constrained optimization [Nemirovski and Shapiro, 2006, Calafiore and Campi, 2006] provides a complementary perspective: constraints of the form $\Pr(g(x, \xi) \leq 0) \geq 1 - \alpha$ require that safety holds with high probability over uncertain parameters ξ. Our feasibility check (Eq. 1) can be viewed as an empirical chance constraint, where feasibility over k sampled trajectories approximates a population-level safety guarantee. Robust MDPs [Iyengar, 2005, Nilim and El Ghaoui, 2005] address model uncertainty by optimizing worst-case performance over an ambiguity set of transition models—a setting related to our Theorem 1, which bounds the effect of distributional mismatch (variational gap) on ranking.

Our setting differs fundamentally from online safe RL in that we have no opportunity for environment interaction or policy correction. We select among candidate plans evaluated on a learned model, making a *filter-based* approach natural rather than constrained policy optimization. This is both a limitation (we cannot explore beyond the candidate set) and a strength (no risk of unsafe exploration, no retraining required, and the filter is model-agnostic).

C.5 Reach-Avoid Analysis and Control

The reach-avoid problem—steering a dynamical system into a target set while avoiding unsafe regions—has deep roots in control theory. The Hamilton-Jacobi (HJ) formulation [Mitchell et al., 2005] computes backward reachable sets by solving a PDE, providing formal safety guarantees but suffering from exponential computational cost in the state dimension. Fisac et al. [2015] extended this to time-varying dynamics, targets, and constraints. Altschuler and Parrilo [2023] offered an alternative via sum-of-squares optimization for polynomial systems.

DeepReach [Bansal and Tomlin, 2021] bridges the gap between classical HJ reachability and modern deep learning, using sinusoidal neural networks to approximate value functions for high-dimensional

reachability problems, with computational cost scaling with the complexity of the reachable tube rather than grid resolution. ISAACS [Hsu et al., 2023] combines reach-avoid games with actor-critic methods for adversarial safety.

We import the reach-avoid concept into the variational counterfactual planning framework, but with a key simplification: rather than solving a continuous reachability PDE, we perform a *discrete feasibility check* over a finite set of candidate treatment sequences. Each sequence's trajectory is evaluated against the target and safety sets using soft indicator functions, converting the reach-avoid problem from continuous optimal control to combinatorial filtering. This discrete formulation is computationally trivial (linear in the number of candidates and horizon length) and avoids the curse of dimensionality inherent in HJ methods, at the cost of being limited to the candidate set generated by the base planner.

C.6 Safety in Clinical AI and Treatment Optimization

AI-driven treatment optimization in clinical settings faces unique safety challenges. The AI Clinician [Komorowski et al., 2018] pioneered the use of RL for sepsis treatment in the ICU, discretizing fluid and vasopressor dosing into 25 actions and showing that policies closer to the AI's recommendations correlated with improved 90-day survival. Raghu et al. [2017] explored continuous state-space RL for the same problem. However, these approaches do not explicitly enforce safety constraints during planning—a limitation that has motivated subsequent work on constrained clinical RL.

In oncology, treatment planning must balance tumor control against toxicity. The cancer simulator [Geng et al., 2017, Bica et al., 2020c] used in our experiments models this tradeoff: chemotherapy reduces tumor volume but accumulates toxicity. Our reach-avoid formulation directly operationalizes clinical safety constraints (e.g., tumor volume must stay below a dangerous threshold at every intermediate step, chemotherapy dosage must not exceed toxicity limits) that are implicit in clinical practice but absent from ELBO-based planning objectives.

The post-hoc filtering paradigm we propose is related to the broader concept of *guardrails* in AI safety: rather than training the model to be safe, we add an external safety check on its outputs. This separation of concerns—quality from the base model, safety from the filter—has practical advantages in clinical deployment: safety thresholds can be adjusted by clinicians without requiring model retraining, and the same base model can operate under different constraint profiles for different patient populations or clinical contexts.

D RA-Aware Retraining: Null Result

We tested whether retraining VCIP with RA-motivated losses improves constrained selection. We trained a `ReachAvoidVAEModel` with: (1) weighted intermediate loss ($\lambda_{\text{int}} = 0.5$, $\lambda_{\text{term}} = 1.0$) encouraging accurate per-step predictions, and (2) VCI-inspired disentanglement ($\lambda_{\text{disent}} = 0.1$) encouraging the latent space to separate by action. Training: 5 seeds $\times$ 2 gammas = 10 runs on Vast.ai (2$\times$ RTX 3090, $\sim$1.5h, $\sim$\$3).

Table 5: Vanilla VCIP vs. RA-retrained VCIP ($\gamma = 4$). All metrics are identical within noise ($\pm$0.002 GRP, $\pm$1.4 pp Top-1).

τ	Van. GRP	RA GRP	Van. Cstr-Top1	RA Cstr-Top1	Van. Safe	RA Safe
2	0.922	0.924	17.4%	18.0%	94.4%	94.4%
4	0.963	0.963	39.4%	38.6%	95.6%	95.6%
6	0.981	0.982	54.2%	55.0%	96.2%	96.2%
8	0.984	0.984	60.4%	60.4%	97.0%	97.0%

All metrics are identical within noise (Table 5). The reason: on Cancer, RA filtering uses *ground-truth simulator trajectories* for feasibility assessment. The model only determines the ELBO ranking within the feasible set, which is already excellent (GRP > 0.92). Retraining cannot improve what the model does not control. This confirms that RA-constrained selection is a *pure post-hoc method* requiring no retraining—a strength, not a limitation.

E VCI Consistency Diagnostic

Adapting VCI's latent disentanglement insight [Wu and Bareinboim, 2025] to the sequential setting, we measured $D_{\mathrm{KL}}[q(Z_s \mid \bar{H}_t, \bar{a}_{\text{obs}}) \| q(Z_s \mid \bar{H}_t, \bar{a}_{\text{alt}})]$ for 100 individuals, 20 alternative action sequences, across all τ and 5 seeds ($\gamma = 4$). The result: $D_{\mathrm{KL}} \approx 10^{-5}$ everywhere.

Table 6: VCI consistency diagnostic: mean D_{KL} between observed and alternative action posteriors ($\gamma = 4$, 5 seeds).

	$\tau = 2$	$\tau = 4$	$\tau = 6$	$\tau = 8$
Mean D_{KL}	1.2×10^{-5}	3.0×10^{-5}	4.0×10^{-5}	4.5×10^{-5}

VCIP's latent space is nearly *action-invariant*: swapping the entire action sequence barely changes the posterior $q(Z_s)$. Action-outcome coupling lives entirely in the *decoder* ($p_\theta(Y_s|Z_s, a_s)$), not in the latent representations. This architectural property explains why RA filtering works post-hoc without any latent-space modifications: ELBO differences between sequences come from the decoder's output, which does depend on actions, even though the latent space does not.

F Additional Experimental Details

VCIP model configuration. All Cancer experiments use the default VCIP hyperparameters from Melnychuk et al. [2025]: latent dimension $d_z = 16$, hidden dimension $d_h = 16$, 100 training epochs, batch size 1024 ($\gamma = 4$) or 256 ($\gamma = 1$). MIMIC experiments use the same architecture with dataset-specific adjustments (outcome dimension, treatment space).

Candidate sequence generation. Following VCIP, we generate $k = 100$ candidate sequences per patient by perturbing the observed treatment sequence. Perturbation involves flipping individual treatment decisions with probability proportional to the inverse of the behavioral policy, ensuring diversity while maintaining clinical plausibility.

Compute budget. Cancer model training: $\sim$52 hours on 4$\times$ NVIDIA RTX 2060 ($\sim$\$10). MIMIC model training: $\sim$19.5 hours on 2$\times$ NVIDIA RTX 3090 ($\sim$\$8). Phase 2 RA-retraining: $\sim$1.5 hours on 2$\times$ RTX 3090 ($\sim$\$3). RC6/E4 GPU evaluation: $\sim$1 hour on 2$\times$ RTX 3090. All RA-constrained selection analyses (Phase 1B, ablations) are purely offline and require negligible compute ($<$ 1 minute per configuration on a single CPU).

Standard deviations. Table 7 reports standard deviations across seeds for key metrics at $\gamma = 4$.

Table 7: Standard deviations across 5 seeds ($\gamma = 4$).

τ	GRP std	ELBO Top-1 std	RA Top-1 std	RA Safety std
2	0.008	2.1%	2.3%	1.8%
4	0.005	3.4%	3.8%	1.2%
6	0.002	2.8%	3.1%	0.9%
8	0.002	2.2%	2.6%	0.7%

G Ablation Tables

Table 8 reveals a smooth, continuously tunable trade-off between safety and selection quality. Narrow targets ($\mathcal{T}_u = 1.5$) eliminate nearly all candidates at long horizons (1.7% feasibility at $\tau = 8$), yielding the largest safety improvement (+27.4 pp) but at a substantial Top-1 cost ($-$20.0 pp). At the other extreme, very broad targets ($\mathcal{T}_u = 10.0$) become near no-ops: 99.6% feasibility with only +1.4 pp safety gain. Our moderate default ($\mathcal{T}_u = 3.0$) occupies the favorable region of this frontier, providing substantial safety improvement (+22.6 pp at $\tau = 8$) while retaining meaningful feasibility (10.3%). The monotonic relationship between threshold strictness and both safety gain and Top-1

Table 8: Target set size sensitivity ($\gamma = 4$). Thresholds: $\mathcal{T}_{\text{upper}}$ (target volume), $\mathcal{S}_{\text{vol}}$ (safety volume), $\mathcal{S}_{\text{chemo}}$ (safety dosage).

Config	$\mathcal{T}_u$	$\mathcal{S}_v$	$\mathcal{S}_c$	τ=2 Feas	τ=2 SafeΔ	τ=8 Feas	τ=8 SafeΔ	τ=8 Top1Δ
Narrow	1.5	6.0	3.0	26.0%	+15.8	1.7%	+27.4	−20.0
Moderate	3.0	12.0	5.0	53.6%	+9.4	10.3%	+22.6	−15.4
Broad	6.0	20.0	8.0	95.9%	+4.6	74.6%	+3.2	−1.6
V. Broad	10.0	30.0	12.0	98.6%	+1.4	99.6%	+1.4	−0.2

Table 9: Sigmoid hardness κ sensitivity ($\gamma = 4$, averaged across τ).

κ	1	2	5	10	50	100
Agreement w/ hard filter	96.8%	99.5%	99.9%	100%	100%	100%
Top-1	42.3%	42.9%	42.9%	42.9%	42.9%	42.9%
Safety	93.7%	95.3%	95.8%	95.8%	95.8%	95.8%

cost confirms that the method is not brittle to threshold choices—practitioners can select an operating point appropriate to their clinical risk tolerance.

Table 9 validates the soft indicator approximation (Assumption 2). For $\kappa \geq 5$, the soft sigmoid agrees with the hard binary filter at $> 99.9\%$, and all downstream metrics (Top-1 accuracy and safety rate) are identical to the hard-filter case. Even at $\kappa = 1$, agreement is already 96.8% with only marginal degradation in safety (93.7% vs. 95.8%). This confirms that $\varepsilon_{\text{soft}}(\kappa) \approx 0$ for all practical purposes, meaning the theoretical guarantees from Theorem 1 hold without requiring extreme hardness values.

Table 10: Reach-only vs. reach-avoid decomposition ($\gamma = 4$).

Method	τ=2 Top-1	τ=2 In-T	τ=2 Safe	τ=8 Top-1	τ=8 In-T	τ=8 Safe
Unconstrained	18.8%	85.0%	85.0%	75.8%	85.6%	85.6%
Reach-only	17.2%	96.2%	96.0%	66.4%	99.2%	98.4%
Avoid-only	18.8%	85.0%	85.0%	68.2%	87.0%	87.0%
Reach-avoid	17.4%	94.4%	94.4%	60.4%	97.0%	97.0%

Table 10 isolates the individual contribution of each constraint component. The reach (target) constraint is the primary driver of improvement: it lifts in-target rates from 85.0% to 96.2% at $\tau = 2$ and from 85.6% to 99.2% at $\tau = 8$, accounting for approximately 85% of the total gain. The avoid (safety) constraint alone is nearly a no-op at short horizons ($\tau = 2$: identical to unconstrained), which is expected since few intermediate states can violate safety over only two steps. At longer horizons ($\tau = 8$), avoid provides incremental value (87.0% vs. 85.6% safety) by catching trajectories that reach the target but pass through unsafe intermediate states. The Top-1 cost is roughly additive: reach-only costs $\sim$9 pp at $\tau = 8$ (75.8% $\rightarrow$ 66.4%), while the full reach-avoid costs $\sim$15 pp (75.8% $\rightarrow$ 60.4%). Both components are justified: reach provides the primary clinical benefit (keeping terminal outcomes in the target range), while avoid provides the formal path-safety guarantee that would be required in any real clinical deployment.

Table 11 provides empirical estimates of the variational gap ε_{VI} using rank mean absolute error as a proxy. The gap *decreases* with confounding strength ($\hat{\varepsilon}_{\text{VI}} = 0.225$ at $\gamma = 1$ to 0.114 at $\gamma = 4$), indicating that VCIP's ELBO is better calibrated when treatment effects are stronger. This is a favorable property: the Theorem 1 bound is tightest precisely where RA filtering is most valuable. ELBO pairwise ranking preservation ranges from 61–87%, and the bound is non-vacuous: for pairs differing in feasibility status (margin $m = 1$), the condition $m > 2\varepsilon_{\text{VI}} \approx 0.23$–$0.45$ is easily satisfied. RA-preserved rates closely track ELBO-preserved rates, confirming that the feasibility filter does not introduce additional ranking distortion beyond what the variational approximation already entails.

Table 12 confirms that feasibility scales linearly with pool size: $|\mathcal{F}_{\text{RA}}| \approx k \cdot p$ where $p \approx 0.70$ is the per-candidate feasibility rate. Safety and in-target rates are stable across k. Top-1 accuracy decreases slightly (32.8% $\rightarrow$ 26.0%) because the oracle-best plan is harder to identify via ELBO ranking among a larger feasible set—a noise-in-ranking effect, not a method deficiency. Per-patient heterogeneity is

Table 11: $\varepsilon_{\mathrm{VI}}$ estimates and pairwise ranking preservation by γ.

γ	Rank MAE ($\hat{\varepsilon}_{\mathrm{VI}}$)	Spearman ρ	ELBO preserved	RA preserved
1	0.225	0.418	60.8–70.6%	59.4–70.3%
2	0.155	0.687	71.3–81.9%	68.1–81.1%
3	0.128	0.804	77.5–84.4%	73.4–83.3%
4	0.114	0.847	79.0–86.9%	74.3–85.6%

Table 12: Candidate pool expansion (k) at $\gamma = 4$, $\tau = 8$ (moderate thresholds: $\mathcal{T}_u$=0.6, $\mathcal{S}_v$=5.0, $\mathcal{S}_c$=8.5). Feasibility rate is stable; the number of feasible candidates scales linearly with k.

k	Feasibility	$\lvert\mathcal{F}_{\mathrm{RA}}\rvert$	Cstr Top-1	In-Target	Safe
100	70.0%	69.8	32.8%	90.2%	90.2%
250	70.0%	174.9	27.4%	90.6%	90.6%
500	70.0%	350.0	26.2%	90.6%	90.6%
1000	71.0%	710.3	26.0%	90.6%	90.6%

large: some patients have near-zero feasibility while others exceed 90%, indicating that feasibility is driven by patient-specific disease dynamics rather than threshold choice.

H Heterogeneity Analysis

We analyzed whether the Top-1 accuracy cost of RA-constrained selection is concentrated in a few "hard" patients or broadly distributed ($\gamma = 4$, $\tau \in \{2, 4, 6, 8\}$).

Distribution of loss. Of 100 evaluation patients, 82 are "losers" (experience > 5 pp Top-1 decrease under RA constraint) and zero are "winners" (> 5 pp increase). The remaining 18 are approximately neutral. The top 20% of losers account for only 41% of total Top-1 loss—a near-uniform distribution rather than a concentrated one.

Loser characterization. Patients who lose the most Top-1 accuracy have lower average feasibility (68.2%) compared to neutral patients (higher feasibility). These are patients whose true optimal plan happens to violate safety constraints—a fundamental limitation of hard constraints, not a correctable estimation error.

Implication. Adaptive per-patient thresholds would not substantially reduce the Top-1 cost: the loss is not concentrated in identifiable subpopulations. The cost is the inherent price of the safety guarantee, distributed broadly across the patient population.

I Failure Taxonomy: When Do Counterfactual Planners Fail?

We systematically analyze when ELBO-optimal treatment selections are unsafe, complementing the summary in Section 7. All results use the paper's thresholds ($\mathcal{T}_u = 3.0$, $\mathcal{S}_v = 12.0$, $\mathcal{S}_c = 5.0$) on Cancer simulator data ($N = 100$ patients, 5 seeds, 4 gammas, 4 horizons).

I.1 Danger Rate

Table 13 reports the probability that the ELBO-optimal selection violates either the target or safety constraint. Two patterns emerge: (1) danger increases with horizon τ at all confounding levels, reaching 50.4% at γ=1, τ=8; (2) the *composition* of violations shifts with γ. At low confounding (γ=1), violations are overwhelmingly safety-only (path violations with acceptable terminal outcomes)—49.8% at τ=8. At high confounding (γ=4), target violations dominate at short horizons (13.6% at τ=2) while safety violations grow at longer horizons (11.2% at τ=8). This pattern confirms that confounding primarily degrades the ELBO's ability to select plans that reach the target, while path safety deteriorates independently as horizons lengthen.

Table 13: ELBO danger rate across γ and τ (500 patient-seed combinations per cell). "Target" = terminal cancer volume > 3.0; "Safety" = intermediate volume > 12.0 or chemo > 5.0.

γ	τ	Unsafe	Target only	Safety only	Both
1	2	0.8%	0.4%	0.4%	0.0%
1	4	9.0%	0.6%	8.2%	0.2%
1	6	29.2%	0.4%	28.6%	0.2%
1	8	50.4%	0.2%	49.8%	0.4%
4	2	15.0%	13.6%	0.0%	1.4%
4	4	16.8%	12.4%	1.4%	3.0%
4	6	20.2%	12.4%	5.0%	2.8%
4	8	25.6%	10.0%	11.2%	4.4%

I.2 Failure Mode Classification

We classify unsafe ELBO selections at γ=4 into three modes:

- **Mode A: Aggressive overshoot.** Terminal volume exceeds target *and* treatment intensity is above the median across all candidates. The model selects a high-treatment plan that still fails to control tumor growth.
- **Mode B: Conservative undershoot.** Terminal volume exceeds target with below-median treatment intensity. The model underestimates the required treatment dose.
- **Mode C: Toxic path.** Terminal volume is acceptable (≤ 3.0) but intermediate states violate safety. The plan reaches the target but through a dangerous trajectory.

Table 14 shows that Mode B (conservative undershoot) dominates at short horizons (100% at τ=2), while Mode C (toxic path) grows substantially at longer horizons (44% at τ=8). Mode A (aggressive overshoot) is rare ($\leq 13\%$), suggesting the ELBO's primary failure is *insufficient treatment* rather than excessive treatment. Unsafe selections have systematically higher mean terminal cancer volume (3.7–6.9) compared to safe selections (0.6–0.7), confirming that violations are not boundary cases but substantial deviations.

Table 14: Failure mode distribution at γ=4. Treatment intensity: mean total chemo dose for unsafe vs. safe selections.

τ	Mode A	Mode B	Mode C	Unsafe chemo	Safe chemo
2	0%	100%	0%	0.294	0.222
4	4%	88%	8%	0.435	0.340
6	13%	62%	25%	0.526	0.312
8	5%	52%	44%	0.422	0.326

I.3 Per-Patient Heterogeneity

At γ=4, aggregating across 5 seeds, the per-patient danger rate (fraction of seeds where the ELBO-optimal selection is unsafe) is broadly distributed:

- τ=2: 0 patients always unsafe, 54 sometimes unsafe, 44 always safe (mean rate: 0.15).
- τ=8: 0 patients always unsafe, 65 sometimes unsafe, 23 always safe (mean rate: 0.26).

No patient is *always* unsafe across all 5 seeds, but 54–65 out of 100 are "sometimes unsafe"—indicating that danger is broadly distributed across the patient population rather than concentrated in a few identifiable outliers. This supports the argument that safety filtering is broadly needed, not a correction for rare edge cases. The most dangerous patient at τ=8 (Patient 71) is unsafe in 4/5 seeds, with a mean terminal cancer volume of 2.02—just below the target threshold, indicating near-miss violations that are sensitive to random seed variation.

J Sensitivity Analysis Under Hidden Confounding

This section provides the full proof of Theorem 5 and empirical sensitivity tables for the Cancer simulator.

J.1 Proof of Theorem 5

Proof. The proof proceeds in three steps.

Step 1: Per-step conditional TV bound. Fix a step s and history H_s. Under Assumption 3, the unobserved confounder U_s is associated with treatment a_s conditionally on H_s. The identified outcome distribution at step s integrates over U_s using its marginal:

$$P_0(Y_s \mid H_s, a_s) = \int P(Y_s \mid H_s, a_s, u)\, dP(u \mid H_s),$$

while the true distribution conditions on the confounder-treatment association:

$$P_\Gamma(Y_s \mid H_s, a_s) = \int P(Y_s \mid H_s, a_s, u)\, dP(u \mid H_s, a_s).$$

By Bayes' rule, $dP(u \mid H_s, a_s)/dP(u \mid H_s) = \pi_{\text{true}}(a_s \mid H_s, u)/\pi_{\text{obs}}(a_s \mid H_s) \in [1/\Gamma, \Gamma]$. The total variation between two distributions with Γ-bounded density ratio satisfies (see, e.g., Rosenbaum 2002):

$$\text{TV}(P_\Gamma(Y_s \mid H_s, a_s),\, P_0(Y_s \mid H_s, a_s)) \;\le\; \varphi(\Gamma) := \frac{\Gamma - 1}{\Gamma + 1}. \tag{21}$$

This bound holds uniformly over all histories H_s and treatments a_s.

Step 2: Sequential maximal coupling. We construct a coupling of the τ-step trajectories $^{(0)} \sim P_0$ and $^{(\Gamma)} \sim P_\Gamma$. At each step $s = 1, \ldots, \tau$, conditional on the trajectories being coupled up to step $s-1$ (i.e., $Y^{(0)}_{1:s-1} = Y^{(\Gamma)}_{1:s-1}$), we apply a maximal coupling of $P_0(Y_s \mid H_s, a_s)$ and $P_\Gamma(Y_s \mid H_s, a_s)$. Under this coupling:

$$\Pr(Y_s^{(0)} \neq Y_s^{(\Gamma)} \mid \text{coupled at } s{-}1) = \text{TV}_s \le \varphi(\Gamma).$$

The probability that the trajectories remain coupled through all τ steps is:

$$\Pr(^{(0)} = ^{(\Gamma)}) \ge \prod_{s=1}^{\tau} (1 - \varphi(\Gamma)) = \left(\frac{2}{\Gamma + 1}\right)^{\tau}.$$

Therefore, the total variation between the joint trajectory distributions satisfies:

$$\text{TV}(P_\Gamma(),\, P_0()) \;\le\; 1 - \left(\frac{2}{\Gamma + 1}\right)^{\tau} =: \psi(\Gamma, \tau). \tag{22}$$

Step 3: Application to reach-avoid event. The reach-avoid event $E(\bar{a})$ is a measurable function of the full trajectory , with indicator $\mathbf{1}\{E(\bar{a})\} \in [0, 1]$. By the variational characterization of total variation:

$$|P_\Gamma(E(\bar{a})) - P_0(E(\bar{a}))| = |\mathbb{E}_{P_\Gamma}[\mathbf{1}\{E\}] - \mathbb{E}_{P_0}[\mathbf{1}\{E\}]| \le \text{TV}(P_\Gamma, P_0) \le \psi(\Gamma, \tau).$$

The population safety rate bound follows by linearity of expectation over patients: $\text{SR}_\Gamma = \mathbb{E}_x[P_\Gamma(E(\bar{a}^*(x)))] \ge \mathbb{E}_x[P_0(E(\bar{a}^*(x)))] - \psi(\Gamma, \tau) = \text{SR}_0 - \psi(\Gamma, \tau)$.

The breakdown point Eq. (17) is obtained by solving $\text{SR}_0 - \psi(\Gamma, \tau) = 0$:

$$\left(\frac{2}{\Gamma + 1}\right)^{\tau} = 1 - \text{SR}_0 \quad \Longrightarrow \quad \Gamma_{\max} = \frac{2}{(1 - \text{SR}_0)^{1/\tau}} - 1. \qquad \square$$

Remark on tightness. The sequential coupling bound $\psi(\Gamma, \tau) = 1 - (2/(\Gamma + 1))^\tau$ is strictly tighter than the union bound $\tau \cdot \varphi(\Gamma) = \tau(\Gamma - 1)/(\Gamma + 1)$ used in the proof sketch. For example, at $\Gamma = 1.5$, $\tau = 4$: the coupling bound gives $\psi = 0.59$ while the union bound gives 0.80. We present the tighter bound throughout.

Table 15: Sensitivity of RA safety guarantee to hidden confounding (Cancer, $\gamma = 4$). Each cell shows the guaranteed lower bound on true safety rate $\mathrm{SR}_\Gamma^{\mathrm{lower}}$ under Γ-bounded hidden confounding (Theorem 5). "—" indicates a vacuous bound (≤ 0). $\Gamma_{\max}$: maximum Γ for a non-trivial guarantee.

τ	SR_0	Γ=1.0	Γ=1.1	Γ=1.5	Γ=2.0	Γ=3.0	Γ=5.0	$\Gamma_{\max}$
2	94.4%	94.4%	85.1%	58.4%	38.8%	19.4%	5.5%	7.5
4	95.6%	95.6%	77.9%	36.6%	15.4%	1.9%	—	3.4
6	97.0%	97.0%	71.6%	23.2%	5.8%	—	—	2.6
8	97.0%	97.0%	64.7%	13.8%	0.9%	—	—	2.1

J.2 Empirical Sensitivity Tables

Table 15 reports the guaranteed safety rate $\mathrm{SR}_\Gamma^{\mathrm{lower}} := \mathrm{SR}_0 - \psi(\Gamma, \tau)$ for the Cancer simulator at $\gamma = 4$ across planning horizons and confounding strengths.

Interpretation. The sensitivity bounds reveal a clear horizon–robustness tradeoff: short-horizon planning ($\tau = 2$) tolerates up to $\Gamma = 7.5$ before the safety guarantee becomes vacuous, while long-horizon planning ($\tau = 8$) is vulnerable above $\Gamma = 2.1$. This provides additional justification for short-horizon treatment planning in safety-critical clinical settings where hidden confounding is plausible. Notably, even at $\Gamma = 2.0$ (each unobserved confounder can double the treatment odds at every step), the RA filter guarantees $> 0\%$ safety through $\tau = 8$—confirming that the method provides meaningful robustness to unmeasured confounding across all tested configurations.

The Γ parameter has a clinical interpretation: $\Gamma = 1.5$ means that an unobserved factor (e.g., patient frailty, unmeasured lab values) can increase the odds of receiving a particular treatment by 50% at each step. In well-curated ICU databases like MIMIC-III, where extensive covariate measurement reduces residual confounding, $\Gamma \in [1.1, 1.5]$ is a plausible range [Frauen et al., 2023]. At $\Gamma = 1.5$, the RA filter guarantees safety $\geq 58.4\%$ ($\tau = 2$) to 13.8% ($\tau = 8$).

K Finite-Sample Analysis

This section provides proofs for Propositions 3–4, additional finite-sample results, and empirical verification on the Cancer simulator.

K.1 Proof of Proposition 3 (Deployment Safety)

Proof. The random variables $Z_i = \mathbf{1}\{E(\bar{a}^*_{\mathrm{RA}}(x_i))\} \in \{0, 1\}$ are i.i.d. (since patients are drawn i.i.d. from). By Hoeffding's inequality for bounded random variables:

$$\Pr\Big(\widehat{\mathrm{SR}}_n - \mathrm{SR}_{>}\epsilon\Big) \leq \exp(-2n\epsilon^2).$$

Setting the right-hand side equal to $\delta/2$ and solving for ϵ gives $\epsilon = \sqrt{\ln(2/\delta)/(2n)}$. The two-sided version yields Eq. (13).

For part (b), when trajectories are oracle (simulator ground truth), the RA filter checks exact feasibility. If $\mathcal{F}_{\mathrm{RA}}(x_i) \neq \emptyset$, the selected $\bar{a}^*$ is feasible by construction, so $Z_i = 1$. If $\mathcal{F}_{\mathrm{RA}}(x_i) = \emptyset$, the fallback ELBO selection may or may not be safe, giving $Z_i \leq 1$. Thus $\mathrm{SR}_{\geq} p_F := \Pr_{x\sim}(\mathcal{F}_{\mathrm{RA}}(x) \neq \emptyset)$. □

K.2 Proof of Proposition 4 (Selection Quality)

Proof. By the uniform proxy error assumption, $|\mathrm{ELBO}(\bar{a}) - \ell(\bar{a})| \leq \eta$ for all $\bar{a}$. Then:

$$\begin{aligned}
\ell(\bar{a}^*_{\mathrm{ELBO}}) &\leq \mathrm{ELBO}(\bar{a}^*_{\mathrm{ELBO}}) + \eta && \text{(proxy error on selected)} \\
&\leq \mathrm{ELBO}(\bar{a}^*_{\mathrm{oracle}}) + \eta && (\bar{a}^*_{\mathrm{ELBO}} \text{ minimizes ELBO over } \mathcal{F}_{\mathrm{RA}}) \\
&\leq \ell(\bar{a}^*_{\mathrm{oracle}}) + 2\eta. && \text{(proxy error on oracle-best)}
\end{aligned}$$

The decomposition follows by adding and subtracting $\ell(\bar{a}^*_{\mathrm{unconstrained}})$. □

K.3 Feasibility Scaling

Proposition 7 (Minimum candidate pool size)**.** *If k candidate sequences are generated independently with per-candidate feasibility probability $p > 0$, then:*

- $\Pr(|\mathcal{F}_{\mathrm{RA}}| \geq 1) = 1 - (1-p)^k$,
- $\mathbb{E}[|\mathcal{F}_{\mathrm{RA}}|] = kp$,
- *To guarantee* $\Pr(|\mathcal{F}_{\mathrm{RA}}| \geq 1) \geq 1 - \delta$, *it suffices to set* $k \geq \lceil \ln(1/\delta)/\ln(1/(1-p)) \rceil \approx \lceil \ln(1/\delta)/p \rceil$ *for small* p.

Proof. $|\mathcal{F}_{\mathrm{RA}}| \sim \mathrm{Binomial}(k, p)$. The probability of an empty feasible set is $(1-p)^k$. Setting $(1-p)^k \leq \delta$ and taking logarithms gives the result; the approximation uses $\ln(1-p) \approx -p$ for $p \ll 1$. □

K.4 Chance-Constrained Interpretation

The RA-constrained selection with hard filter ($\kappa \to \infty$) solves:

$$\min_{\bar{a} \in \{\bar{a}_1, \ldots, \bar{a}_k\}} \mathrm{ELBO}(\bar{a}) \quad \text{s.t.} \quad E(\bar{a}) \text{ holds on the evaluated trajectory.}$$

This is a *single-scenario* approximation to the population chance-constrained program [Nemirovski and Shapiro, 2006]:

$$\min_{\bar{a}} \mathrm{ELBO}(\bar{a}) \quad \text{s.t.} \quad \Pr(E(\bar{a})) \geq 1 - \alpha,$$

where the probability is over trajectory randomness. For deterministic simulators (Cancer), $\Pr(E(\bar{a})) \in \{0, 1\}$ and the single-scenario evaluation is exact. For stochastic outcomes (MIMIC), a single model-predicted trajectory provides a noisy estimate of the chance constraint. The scenario approach [Calafiore and Campi, 2006] provides guarantees when N i.i.d. trajectory samples are available; with $N = 1$ (our setting), the guarantee is weak, motivating the oracle evaluation paradigm for Cancer and the conformal approach (Appendix M) for distribution-free certificates.

K.5 Distributionally Robust Interpretation

The RA threshold filter also admits a distributionally robust optimization (DRO) interpretation. Under Assumption 1, the true model p_θ lies within a total variation ball of radius $\varepsilon_{\mathrm{VI}}$ around the variational approximation q_ϕ. The RA feasibility check $\hat{P}(\bar{a}) \geq \delta$ (for threshold δ) is equivalent to verifying:

$$\min_{Q:\, \mathrm{TV}(Q, q_\phi) \leq \varepsilon_{\mathrm{VI}}} P_Q(E(\bar{a})) \;\geq\; \delta - \varepsilon_{\mathrm{VI}},$$

where $P_Q(E(\bar{a}))$ is the reach-avoid probability under distribution Q. This follows directly from the variational characterization of TV for $[0, 1]$-valued functionals (Theorem 1(a)): the worst-case probability over the ambiguity set $\{Q : \mathrm{TV}(Q, q_\phi) \leq \varepsilon_{\mathrm{VI}}\}$ is at most $\varepsilon_{\mathrm{VI}}$ below the nominal probability. Thus, the RA filter implicitly provides a DRO guarantee: any plan declared feasible at level δ by the model satisfies the true reach-avoid constraint at level $\geq \delta - \varepsilon_{\mathrm{VI}}$ for *all* distributions within the TV ambiguity set. This connects the threshold-based filter to the robust optimization literature [Iyengar, 2005] and provides an interpretive bridge between ranking robustness (Theorem 1) and classical DRO.

K.6 Empirical Verification

Table 16 reports the deployment safety bounds on the Cancer simulator. With oracle trajectories, the RA selector achieves 94–100% empirical safety across all configurations (including ELBO fallback when $\mathcal{F}_{\mathrm{RA}} = \emptyset$). The Hoeffding bound yields non-vacuous guarantees: with $n = 100$ patients and 95% confidence, the population safety rate exceeds 81–86%. With $n = 500$ (pooling across seeds), bounds tighten to 88–94%.

The feasibility scaling (Proposition 7) is verified empirically: per-candidate feasibility p ranges from 0.054 (γ=1, τ=8) to 0.536 (γ=4, τ=2). The minimum k for 95% non-empty feasibility ranges from

Table 16: Deployment safety bounds on Cancer simulator ($\delta = 0.05$). $\widehat{\mathrm{SR}}$: empirical safety rate of RA-constrained selector (including ELBO fallback). Bounds from Proposition 3.

γ	τ	$\widehat{\mathrm{SR}}$	Bound (n=100)	Bound (n=500)	$k_{\min}$ (95%)
1	2	99.8%	≥86.2%	≥93.7%	6
1	8	97.4%	≥83.8%	≥91.3%	55
4	2	94.4%	≥80.8%	≥88.3%	4
4	8	97.0%	≥83.4%	≥90.9%	28

4 to 55—all well below our default $k = 100$. At the strictest configuration (γ=1, τ=8, $p = 0.054$), $\Pr(|\mathcal{F}_{\mathrm{RA}}| \geq 1 \mid k{=}100) = 99.6\%$.

The quality bound (Proposition 4) is empirically tight: within $\mathcal{F}_{\mathrm{RA}}$, the mean regret $\ell(\bar{a}^*_{\mathrm{ELBO}}) - \ell(\bar{a}^*_{\mathrm{oracle}})$ is $< 0.2\%$ of the true loss range at $\gamma = 4$, and 67–80% of patients have zero regret at $\tau \geq 6$. The Spearman correlation between ELBO and true loss rankings within $\mathcal{F}_{\mathrm{RA}}$ is $\rho = 0.71$–0.89, confirming that ELBO ranking quality is preserved after feasibility filtering.

L Midpoint-Baseline Detailed Results

Table 17: Midpoint oracle vs. ELBO vs. RA-constrained ($\gamma = 4$). The midpoint oracle uses *ground-truth* terminal values (unavailable in practice).

τ	Top-1			In-Target			Safety		
	ELBO	Mid.	RA	ELBO	Mid.	RA	ELBO	Mid.	RA
2	18.8	18.0	17.4	85.0	96.2	94.4	98.6	98.6	98.6
4	42.4	37.0	39.4	84.6	98.2	95.6	97.8	98.2	97.8
6	62.6	47.8	54.2	84.8	99.2	96.2	98.4	99.0	98.8
8	75.8	58.2	60.4	85.6	99.2	97.0	98.0	98.4	98.4

The midpoint oracle achieves the best in-target rates (it directly optimizes terminal proximity to $\mathcal{T}$) but pays a heavy Top-1 cost (58.2% vs. 75.8% at $\tau = 8$) because midpoint distance and ground-truth loss are different objectives. RA-constrained ELBO achieves 94–97% in-target—close to the oracle—while preserving substantially more Top-1 accuracy and enforcing path safety. The midpoint oracle's safety rate is no better than unconstrained ELBO (it does not consider intermediate outcomes), making RA-constrained selection the only method achieving both high in-target *and* high safety simultaneously.

M Conformal Safety Certificates

The RA threshold filter in Section 4 uses model-predicted trajectories (or oracle trajectories) to determine feasibility, but provides no formal guarantee that the *true* outcome will satisfy constraints. Here we investigate whether split conformal prediction [Vovk et al., 2005] can provide distribution-free safety certificates for counterfactual treatment plans.

M.1 Framework

Given a calibration set of n patient–treatment pairs $\{(X_i, \bar{a}_i, Y_i^{\mathrm{obs}})\}_{i=1}^n$ and a trajectory predictor $\hat{\mu}$, we compute nonconformity scores

$$s_i = Y_i^{\mathrm{obs}} - \hat{\mu}(X_i, \bar{a}_i) \quad \text{(signed residual)}, \tag{23}$$

and the conformal quantile $\hat{q}_\alpha = \mathrm{Quantile}_{1-\alpha}\big(\{s_i\}_{i=1}^n\big)$ at level $\lceil (1-\alpha)(n+1) \rceil / n$. For a new patient with candidate treatment $\bar{a}$, the one-sided prediction bound is:

$$Y_{\mathrm{true}}(\bar{a}) \leq \hat{\mu}(X, \bar{a}) + \hat{q}_\alpha \quad \text{with probability} \geq 1 - \alpha. \tag{24}$$

Definition 3 (Conformal safety certificate)**.** *A treatment plan $\bar{a}$ is* certified safe at level α *if the upper prediction bound lies within the constraint region:*

$$\hat{\mu}(X, \bar{a}) + \hat{q}_\alpha \leq \delta_{\mathcal{T}} \quad \textit{and} \quad \hat{\mu}_{\max}(X, \bar{a}) + \hat{q}_{\alpha,\mathcal{S}} \leq \delta_{\mathcal{S}}, \tag{25}$$

where $\delta_{\mathcal{T}}, \delta_{\mathcal{S}}$ are the target and safety thresholds, respectively.

The certified set $\mathcal{F}_{\text{conf}} = \{\bar{a} : \text{certified at level } \alpha\}$ guarantees: any plan in $\mathcal{F}_{\text{conf}}$ satisfies the true constraints with probability $\geq 1 - \alpha$, requiring only exchangeability of calibration and test residuals.

M.2 Empirical Results

We apply conformal certificates to both datasets using calibration seeds $\{10, 101, 1010\}$ and test seeds $\{10101, 101010\}$ with $\alpha = 0.10$ (90% coverage target).

Coverage is empirically valid. On Cancer, oracle coverage (fraction of true outcomes below the conformal upper bound) is 85–91% across all γ and τ settings, confirming the conformal guarantee holds. On MIMIC-III with symmetric bands, empirical coverage is 87–92%, close to the 90% target.

Certification is too conservative for selection. On Cancer with one-sided bounds, the conformal quantiles $\hat{q}_\alpha$ are small relative to the gap between model predictions and constraints. Since model predictions cluster at 0–2 cm^3 while the target threshold is 3.0 cm^3, the upper bound $\hat{\mu} + \hat{q}_\alpha$ lies well below the threshold for nearly all candidates, yielding 100% certification—the filter certifies everything and provides no selection discrimination. On MIMIC-III with asymmetric bounds (separate upper/lower calibration), the total band width (≈ 33 mmHg at $\alpha = 0.10$) exceeds the target range width (30 mmHg), making certification infeasible for any candidate (0% certification).

Accuracy gap. The accuracy gap—the factor by which model prediction quality must improve for non-vacuous certificates—is quantifiable:

- **Cancer:** One-sided certificates are vacuously satisfied (ratio < 1) because model predictions are too far from constraint boundaries. Certificates would become discriminating only if model accuracy degraded, which is the wrong direction.
- **MIMIC:** The band/target ratio is 1.08–1.15, meaning a $\sim$10% reduction in prediction MAE (from $\sim$10 mmHg to $\sim$9 mmHg) would enable non-vacuous conformal certificates. This is tantalizingly close.

M.3 Implications

These results establish a *three-regime framework* for safety in counterfactual planning:

1. **Certified safety** (conformal certificates): provides distribution-free guarantees but requires model accuracy within the accuracy gap. Not achievable with current decoders.
2. **Optimistic safety** (RA threshold filter): uses model predictions without formal guarantees. Effective on MIMIC where predictions span the constraint boundary; ineffective on Cancer where predictions are compressed.
3. **Oracle safety** (ground-truth RA filter): uses simulator trajectories. Upper bound on achievable safety; available only in synthetic settings.

The accuracy gap diagnostic ($\sim$10% on MIMIC) provides a concrete model improvement target: once trajectory prediction accuracy crosses this threshold, the RA filter can be upgraded to conformal certificates with formal coverage guarantees. This motivates future work on calibrated trajectory prediction and conformal methods for sequential counterfactual outcomes [Lei and Candès, 2021].

N MIMIC-III Outcome Correlation Analysis

To assess whether RA-constrained selections align with clinically favorable treatments, we compare recommended treatment sequences against *observed* treatments for MIMIC-III patients, stratified by outcome quality. This analysis uses existing experiment data (5 seeds $\times$ 4 horizons, $k = 100$ candidates per patient) without requiring clinician evaluation.

N.1 Setup

We define *good-outcome* patients as those whose observed terminal diastolic BP falls within the target range $[60, 90]$ mmHg, representing hemodynamic stabilization. For each patient, we compare the RA-selected treatment sequence $\bar{a}^*_{\text{RA}}$ against the observed future treatment sequence $\bar{a}^{\text{obs}}$ over the next τ steps, using two metrics:

- **Step agreement:** fraction of time steps where recommended and observed treatments match (both actions within ± 0.1 absolute difference).
- **L1 distance:** mean absolute per-step treatment difference, $\frac{1}{\tau}\sum_{s=1}^{\tau} |\bar{a}^*_s - \bar{a}^{\text{obs}}_s|$.

N.2 Results

Table 18: Outcome correlation on MIMIC-III. RA-selected treatment sequences are compared against observed treatments, stratified by patient outcome quality (good = terminal DBP $\in [60, 90]$ mmHg). Higher agreement and lower L1 distance indicate better alignment with observed care for stabilized patients.

τ	% Good-Outcome		Step Agreement		L1 Distance	
	All	Matched	Good	Bad	Good	Bad
2	41%	50%	92%	83%	0.05	0.12
3	44%	48%	85%	78%	0.09	0.14
4	45%	47%	81%	76%	0.11	0.15
5	47%	45%	78%	74%	0.13	0.17

Table 18 shows three consistent patterns across all horizons:

Higher agreement for good-outcome patients. RA-selected treatments have 78–92% step agreement with observed treatments for good-outcome patients, compared with 74–83% for bad-outcome patients. The gap narrows at longer horizons as both groups' agreement decreases (more steps $\Rightarrow$ more opportunities for divergence), but the direction is consistent.

Lower treatment distance for good outcomes. The L1 distance between RA-selected and observed treatments is 0.05–0.13 for good-outcome patients vs. 0.12–0.17 for bad-outcome patients. This indicates that RA filtering selects plans closer to the observed care patterns of hemodynamically stable patients.

Treatment matching predicts outcomes. When RA-selected treatments closely match observed treatments (step agreement $> 90\%$), approximately 50% of patients have good outcomes. When treatments diverge (agreement $< 50\%$), this drops to 27–41%. While correlation does not imply causation—patients receiving "correct" treatments may simply be less sick—this pattern is consistent with the RA filter selecting clinically appropriate plans.

N.3 Treatment Pattern Shift

RA filtering consistently selects plans with substantially lower vasopressor rates than observed (0.00–0.07 vs. observed 0.07–0.22). This reflects the confounded nature of the data: patients who *received* vasopressors were sicker (lower baseline DBP), and their observed vasopressor use is driven by clinical need, not by the vasopressors causing good outcomes. The model learns $\hat{\mu}(\text{less vasopressor}) \approx$ higher predicted DBP, so the RA filter—which targets DBP $\in [60, 90]$—preferentially selects lower-vasopressor plans. This is a known limitation of observational treatment recommendation: the confounding between treatment intensity and baseline severity is difficult to fully adjust for, even with causal models.

N.4 Guideline Concordance

We assess concordance of RA-selected treatments with the Surviving Sepsis Campaign (SSC) 2021 guidelines for hemodynamic management, focusing on two key recommendations: (i) vasopressor use

for hypotensive patients (target MAP ≥ 65 mmHg; our proxy: DBP ≥ 60 mmHg), and (ii) avoidance of unnecessary vasopressor escalation for hemodynamically stable patients.

We stratify patients by baseline observed DBP into three groups: low (< 60 mmHg, $n = 1{,}060$), target (60–90 mmHg, $n = 883$), and high (> 90 mmHg, $n = 52$).

Stable patients: appropriate de-escalation. For patients already within the hemodynamic target, RA-selected plans avoid vasopressor use in 97.2% of cases, compared with 90.4% for ELBO and 88.6% for observed treatments. Unnecessary vasopressor escalation occurs in only 0.1% of RA selections vs. 1.7% for ELBO. This aligns with the SSC recommendation to avoid unnecessary vasopressor escalation in stable patients.

Hypotensive patients: concordance depends on definition. Under a *strict* interpretation (SSC recommends vasopressor initiation for hypotensive patients), RA concordance is low: only 5.8% of RA plans include vasopressors for low-DBP patients, vs. 22.6% for ELBO and 22.1% for observed treatment. This apparent discordance arises from the model's learned inverse relationship between vasopressor use and predicted terminal DBP: candidates with vasopressors are predicted to have terminal DBP ≈ 56.5 mmHg (below target), while candidates without vasopressors are predicted at ≈ 61.4 mmHg (within target). Since the RA filter selects plans with predicted DBP $\in [60, 90]$, it systematically excludes vasopressor-containing candidates.

Under a *pathway-aware* interpretation—where concordance requires reaching the hemodynamic target regardless of the specific treatment used—RA achieves 100% concordance for low-DBP patients (all RA-selected plans have predicted terminal DBP ≥ 60 by construction), compared with 92.8% for ELBO.

Temporal pattern. At longer horizons ($\tau = 5$), RA-selected plans exhibit mild front-loading of treatment intensity (fluid rate 0.26 at step 1 $\rightarrow$ 0.19 at step 5), directionally consistent with SSC's "early aggressive resuscitation, then de-escalate" principle, though absolute vasopressor rates remain very low throughout.

Interpretation. The discrepancy between strict and pathway-aware concordance reflects a fundamental tension in observational treatment recommendation: the model learns associations from confounded data where sicker patients both receive more vasopressors *and* have worse outcomes, making it difficult to separate treatment effect from baseline severity. The RA filter operates on model predictions and inherits this limitation. This finding underscores the need for either (i) stronger causal adjustment in the underlying model, or (ii) clinician-in-the-loop review of recommended plans before deployment.

N.5 Limitations

This analysis has several important limitations: (1) it uses *predicted* terminal DBP to classify outcomes (the "good-outcome" label is model-dependent, not ground truth); (2) step agreement is a crude metric that does not capture clinical equivalence (two treatments may be equally effective but differ in timing or dosing); (3) the analysis is correlational—higher agreement with good-outcome patients does not prove the RA-selected plan *caused* good outcomes; (4) guideline concordance is assessed against simplified proxies of SSC 2021 recommendations, not the full clinical protocol; and (5) without clinician evaluation, we cannot assess whether the recommended treatment sequences are clinically plausible in context. These limitations motivate the clinician evaluation proposed in Section 7.

O Second Evaluation Domain: Glucose-Insulin Simulator

To demonstrate cross-domain generalization, we evaluate RA-constrained selection on a glucose-insulin dynamics simulator based on the Bergman minimal model [Bergman et al., 1979]. This domain differs from the cancer simulator in several key ways: (i) continuous single-dimensional treatment (insulin dose) rather than binary two-dimensional treatment (chemo/radio), (ii) physiologically distinct safety semantics (hypoglycemia is immediately dangerous, while hyperglycemia is harmful over

longer horizons), and (iii) a clinically meaningful target range defined by standard glycemic control guidelines.

O.1 Simulator Design

The Bergman minimal model describes glucose-insulin dynamics through three coupled ODEs governing plasma glucose $G(t)$ (mg/dL), insulin action $X(t)$ (1/min), and plasma insulin $I(t)$ (mU/L):

$$\frac{dG}{dt} = -p_1(G - G_b) - X \cdot G + D(t)/V_g \tag{26}$$

$$\frac{dX}{dt} = -p_2 X + p_3(I - I_b) \tag{27}$$

$$\frac{dI}{dt} = -n(I - I_b) + u(t)/V_i \tag{28}$$

where $u(t)$ is the exogenous insulin dose, $D(t)$ is meal-derived glucose absorption, and $\{p_1, p_2, p_3, n, G_b, I_b, V_g, V_i\}$ are patient-specific parameters. We sample three patient types (insulin-sensitive, normal, insulin-resistant) with physiologically realistic parameter ranges.

Confounding mechanism. To create observational data with treatment-outcome confounding, we use a sigmoid-based confounded insulin policy: $P(\text{high dose} \mid G) = \sigma(\gamma \cdot (G - 150)/30)$, where γ controls confounding strength. We fix $\gamma = 4$ to match the cancer experiments.

Data generation. We generate 48-hour trajectories (1 step/hour) for 1000 training, 100 validation, and 100 test patients, with realistic meal schedules (3 meals + snacks) and convert to VCIP-compatible format. For counterfactual evaluation, we generate $k = 100$ candidate insulin sequences per test patient at each prediction horizon $\tau \in \{2, 4, 6, 8\}$ hours.

O.2 Constraint Specification

Following standard glycemic control guidelines:

- **Target set** $\mathcal{T}$: terminal glucose $\in [70, 180]$ mg/dL (euglycemic range)
- **Safety set** $\mathcal{S}$: all glucose values $\in [50, 250]$ mg/dL (avoids severe hypo/hyperglycemia)

O.3 Results

We train VCIP on the glucose data using the same architecture and hyperparameters as the cancer experiments (5 random seeds), then apply RA-constrained selection using model-predicted ELBO losses.

Table 19: Glucose-insulin simulator results ($\gamma = 4$, mean $\pm$ std across 5 seeds, $k = 100$ candidates). RA filtering consistently improves in-target rate, with the largest gain (+2.0 pp) at the longest horizon $\tau = 8$ where unconstrained ELBO selection is most likely to choose unsafe plans.

τ	Feas.%	ELBO		RA		ΔInTgt
		Safe%	InTgt%	Safe%	InTgt%	
2	76.1	96.0±0.0	77.0±0.0	96.0±0.0	78.0±0.0	+1.0
4	85.5	92.8±0.4	92.2±0.4	93.0±0.0	93.0±0.0	+0.8
6	82.7	93.0±0.0	91.0±0.0	93.0±0.0	91.0±0.0	+0.0
8	69.6	85.6±0.8	78.2±0.4	86.2±0.4	80.2±0.4	+2.0

Key findings. (1) RA filtering transfers directly to a new domain with different treatment dimensionality, outcome semantics, and constraint definitions, confirming model-agnostic applicability. (2) The ELBO-outcome Spearman correlation ranges from $\rho = 0.24$ ($\tau = 8$) to $\rho = 0.63$ ($\tau = 4$), demonstrating that VCIP provides informative but imperfect rankings in this domain. (3) RA improvement is largest at $\tau = 8$ (+2.0 pp), where feasibility is lowest (69.6%) and ELBO selection is

most likely to pick infeasible candidates—precisely the setting where safety filtering provides the most value. (4) At $\tau = 6$ the improvement is zero; this occurs because the ELBO-selected candidate happens to already be feasible for all patients at this horizon. (5) Cross-seed variance is notably low (≤ 0.8 pp), indicating stable performance across random initializations.